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Question 1

Not answered

Marked out of 3.00

Drug overdose

Please identify the drug that was most likely given in the following scenarios.

1. Miss Carlson is a 32-year-old woman who has brought into the Emergency Department comatose and unresponsive. Ten minutes after she is given a drug she jumps up off the bed and storms out of the department. ✖
2. Mr Davies is a 19 year old man who has been brought to the Emergency Department by friends after consuming 56 Paracetamol tablets following the end of a relationship. He is admitted and given an infusion. ✖
3. Mrs Edwards is a 40-year-old woman who has been admitted for an appendectomy. On her return from theatre she is noted to have a spontaneous respiratory rate of 6 breaths per minute. ✖

Explanation: 1) An abrupt return to consciousness following administration of a medication, particularly in a patient who immediately leaves the department, suggests opiate overdose and subsequent use of naloxone. 2) Parvolex (N-acetylcysteine) is the recommended treatment for paracetamol overdose. When paracetamol is taken in large quantities, a minor metabolite called N-acetyl-p-benzoquinone imine (NAPQI) accumulates within the body. It is normally conjugated by glutathione, but when taken in excess, the body's glutathione reserves are not sufficient to inactivate the toxic NAPQI. This metabolite is then free to react with key hepatic enzymes, thereby damaging liver cells. This may lead to severe liver damage and even death by acute liver failure. N-acetylcysteine acts to maintain or replenish depleted glutathione reserves in the liver and enhance non-toxic metabolism of paracetamol. 3) Flumazenil is used for patients with benzodiazepine overdose or excess. It acts via competitive inhibition at the benzodiazepine-binding site on the GABAA receptor. Benzodiazepines are often used during surgical procedures and these can cause continued respiratory depression in some patients.

Question 2

Not answered

Marked out of 3.00

Treatment of alcoholism.

Which of the following treatments would be most appropriate for each of the following patients?

1. Mr Fletcher is a 34-year-old man who is bingeing on alcohol and cocaine once or twice weekly. He has attended outpatient clinic as he is thinking about stopping. ❌
2. Miss Gaskell is a 47-year-old woman who has a history of alcohol dependence. She successfully completed a community detoxification one week ago and is engaged with a treatment programme. She is struggling with cravings. ❌
3. Mr Gosh completed alcohol detoxification programme in the hospital and discharged on a medication. He has marked nausea and vomiting when alcohol was consumed. ❌

Explanation: 1) Motivational interviewing is the most appropriate intervention to move patients from considering change to enacting and sustaining it. The stages of change are: precontemplation, contemplation, preparation, maintenance, and relapse. This patient would not benefit from medication as he is not physiologically dependent. 2) Acamprosate has an established indication in reducing cravings in newly abstinent patients who have been dependent upon alcohol. Naltrexone is being studied but is not as widely used. 3) Disulfiram induces marked nausea and flushing if alcohol is consumed due to antagonism of aldehyde dehydrogenase.

Question 3

Not answered

Marked out of 3.00

Statistical tests

Which of the following statistical tests would be most appropriate for the following circumstances?

1. Comparing the rates of relapse for two groups of patients with schizophrenia receiving different treatments. 
- ✖
2. Comparing the mean BMI for two groups of patients who are either receiving Olanzapine or Risperidone.  ✖
3. Comparing the risk of developing schizophrenia in populations who either have a history of cannabis use or those who have never been exposed to it.  ✖

Explanation: 1) As rates refer to a categorical outcome (relapse or no relapse, chi-square test can be used for the comparison 2) The unpaired t-test is used when two separate sets of independent samples are obtained, one from each of the two populations being compared. 3) The Chi square test can be used to compare the two risks. We can also use logistic regression with exposure to cannabis as an independent variable, and the binary outcome of schizophrenia as the dependent variable. Epidemiologists usually prefer regression models to chi-square tests so that other factors can also be controlled/adjusted for.

Correct Answer is :

1. Comparing the rates of relapse for two groups of patients with schizophrenia receiving different treatments. - Chi square
2. Comparing the mean BMI for two groups of patients who are either receiving Olanzapine or Risperidone. - Unpaired T test
3. Comparing the risk of developing schizophrenia in populations who either have a history of cannabis use or those who have never been exposed to it. - Chi square

Question 4

Not answered

Marked out of 4.00

Validity and Reliability

Match the following scenarios to their description of validity or reliability:

1. Multisource feedback in for doctors in Foundation Year 2 (F2) was in line with multisource feedback in Foundation Year 1 (F1)



2. Two respondents on multisource feedback gave similar positive reports on the same F1 trainee.



3. Two radiologists reported the same degree of hippocampal atrophy and this was associated with a diagnosis of Alzheimer's dementia



4. A new screening questionnaire for depression was being evaluated and compared to the gold standard that was administered at the same time to see if it could still diagnose depression with fewer items or questions.



Explanation: Inter-rater reliability is used to assess the degree to which different raters/observers give consistent estimates of the same phenomenon. Having good test re-test reliability signifies the internal validity of a test and ensures that the measurements obtained in one sitting are both representative and stable over time. Concurrent validity is a measure of how well a particular test correlates with a previously validated measure. Predictive validity involves testing a group of subjects for a certain construct, and then comparing them with results obtained at some point in the future. Construct validity refers to whether a scale or test measures the construct adequately. It is a broader concept and is best opted for if other options are not specific enough. (Ref: Murphy KR, Davidshofer CO. Psychological Testing: Principles and Applications. 6th ed. Upper Saddle River, N.J.: Pearson/Prentice Hall; 2005)

Correct Answer is :

1. Multisource feedback in for doctors in Foundation Year 2 (F2) was in line with multisource feedback in Foundation Year 1 (F1) - Predictive validity
2. Two respondents on multisource feedback gave similar positive reports on the same F1 trainee. - Inter-rater reliability
3. Two radiologists reported the same degree of hippocampal atrophy and this was associated with a diagnosis of Alzheimer's dementia -

Inter-rater reliability

4. A new screening questionnaire for depression was being evaluated and compared to the gold standard that was administered at the same time to see if it could still diagnose depression with fewer items or questions. - Concurrent validity



Question 5

Not answered

Marked out of 3.00

Evaluating a diagnostic test for clinical use

Answer the following questions on the basis of the information provided in the précis. A new diagnostic test is being introduced to detect schizophrenia, and a study reporting data on comparison with gold standard DSM-5 based consensus criteria is published evaluating 400 outpatient subjects in total. The new scale indicated schizophrenia as a diagnosis for 40 out of the 400 patients while the gold standard procedure identified schizophrenia in only 24 out of those 40. 8 patients identified to have schizophrenia using the gold standard test were missed by the new test.

1. What is the positive predictive value of the new test? ✖

2. What is the sensitivity of the new test? ✖

3. What is the specificity of the new test? ✖

Explanation: Let us first parse out the numbers provided to us.

- "evaluating 400 outpatient subjects in total" - This refers to the final TOTAL box
- "The new scale indicated schizophrenia as a diagnosis for 40 out of the 400 patients" - This refers to the TOTAL test positives box
- "the gold standard procedure identified schizophrenia in only 24 out of those 40" - This refers to the TRUE POSITIVE box
- "8 patients identified to have schizophrenia using the gold standard test were missed by the new test" - This refers to the FALSE NEGATIVE box

Using the information provided, please create the following table

(Note: italicized font in this table refers to numbers that we computed, and not provided in the question)

	DSM +VE Schizophrenia	DSM -VE NO Schizophrenia	TOTAL
Test POSITIVE	24	16	40
Test Negative	8	352	360

TOTAL	32	368	400
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Now,

PPV = true positives/ total positives expressed as a percentage. $PPV = 24/40 = 60\%$

Sensitivity = true positive/ total diseased = $24/32 = 75\%$

Specificity = true negatives/ total non-diseased = $352/368 = 95.7\%$

Correct Answer is :

1. What is the positive predictive value of the new test? - 60%
2. What is the sensitivity of the new test? - 75%
3. What is the specificity of the new test? - 95.7%



Question 6

Not answered

Marked out of 5.00

Childhood disorders

Which of the following diagnoses are most appropriate for each of the below scenarios?

1. Kate Forlock is a 2-year-old girl who was adopted into her current family around 9 months ago. Her mother has raised concerns with the health visitor that Kate does not seem to respond to affection, does not seek out cuddles and seems ambivalent to her presence. She is developing normally otherwise. ❌
2. Colin Eames is a 4-year-old boy who has limited speech. He cannot use sentences and, for example, only uses the word "drink" when he wants water. He becomes very upset when he is away from home, especially in places that are noisy. He does not show much interest in playing with adults or other children. ❌
3. Holly Ward is a 9-year-old girl whose teachers have raised concerns about her. She has a good vocabulary but cannot write sentences of longer than 6 words or read books, which would normally be suitable for children 3-4 years, her junior. She has started to have angry outbursts in class. On interview, she presents as bright and is able to answer your questions clearly. ❌
4. Simon Lovell is a 10-year-old boy who struggles to concentrate at school and becomes upset in loud environments. However, he is happy at home and can produce excellent work at home without difficulty. ❌
5. Laura Tierney is a 9-year-old girl who is doing well at school and has plenty of friends. However, she is getting upset as she is unable to ride her bike and tie her shoelaces, unlike her peers. ❌

Explanation: 1) This is likely to be reactive attachment disorder. This arises from a failure to form normal attachments to primary caregivers in early childhood, often due to early experiences of neglect, abuse, abrupt separation from caregivers between the ages of six months and three years, frequent change of caregivers, or a lack of caregiver responsiveness to a child's communicative efforts. It is characterised by inappropriate social interactions, which are often inhibited or limited in scope. 2) This is likely to represent an autistic spectrum disorder, which is characterised by limited language development, impaired social skills and restricted and repetitive interests. 3) This is likely to be dyslexia, characterised by limited reading skills, which then impair writing ability. Her angry outbursts are likely the result of her frustrations over this. 4) This is likely to be an auditory processing disorder in which an individual has difficulty recognising and interpreting sounds

properly. In children this can impair their concentration and lead to them becoming overwhelmed and upset in loud places. 5) This is likely to represent dyspraxia, in which an individual struggles to co-ordinate movements. It can be associated with other developmental disorders but can occur alone.

Correct Answer is :

1. Kate Forlock is a 2-year-old girl who was adopted into her current family around 9 months ago. Her mother has raised concerns with the health visitor that Kate does not seem to respond to affection, does not seek out cuddles and seems ambivalent to her presence. She is developing normally otherwise. - Reactive attachment disorder
2. Colin Eames is a 4-year-old boy who has limited speech. He cannot use sentences and, for example, only uses the word "drink" when he wants water. He becomes very upset when he is away from home, especially in places that are noisy. He does not show much interest in playing with adults or other children. - Autism
3. Holly Ward is a 9-year-old girl whose teachers have raised concerns about her. She has a good vocabulary but cannot write sentences of longer than 6 words or read books, which would normally be suitable for children 3-4 years, her junior. She has started to have angry outbursts in class. On interview, she presents as bright and is able to answer your questions clearly. - Dyslexia
4. Simon Lovell is a 10-year-old boy who struggles to concentrate at school and becomes upset in loud environments. However, he is happy at home and can produce excellent work at home without difficulty. - Auditory processing disorder
5. Laura Tierney is a 9-year-old girl who is doing well at school and has plenty of friends. However, she is getting upset as she is unable to ride her bike and tie her shoelaces, unlike her peers. - Dyspraxia



Question 7

Not answered

Marked out of 3.00

Forensic setting

Which of the following setting would be most appropriate for the patients described in the below scenarios?

1. Mr Adam Turner is a 34-year-old man who is currently serving a prison term for murder. He has developed a psychotic episode and is presenting as unsettled and 4 prison officers are involved with any direct interactions with him. ✖
2. Mr Brian Rogers is a 26-year-old man who has served a previous prison term for burglary and who has now been arrested for shoplifting. The police raise concerns about his mental state and on assessment he is found to be suffering from an episode of mania. He lacks insight and is unwilling to take any medication. ✖
3. Mr Vincent George is a 46-year-old man who is currently on remand in prison awaiting trial for armed robbery. He collapses on the prison wing and is found to be hypotensive, tachycardic and with marked pallor. ✖

Explanation: 1) Due to this individual's degree of agitation and his previous offending, he can only be managed in a high secure adult unit at present. 2) This individual will likely require admission to a Psychiatric Intensive Care Unit as he is presenting with mania and would not be manageable on a general adult ward. He would not require higher security as his previous offending has been acquisitive rather than violent. 3) This individual would require immediate and comprehensive physical evaluation in a general hospital. When this is required, inmates are generally accompanied by at least 2 prison officers and kept in restraints throughout.

Correct Answer is :

1. Mr Adam Turner is a 34-year-old man who is currently serving a prison term for murder. He has developed a psychotic episode and is presenting as unsettled and 4 prison officers are involved with any direct interactions with him. - High secure adult ward
2. Mr Brian Rogers is a 26-year-old man who has served a previous prison term for burglary and who has now been arrested for shoplifting. The police raise concerns about his mental state and on assessment he is found to be suffering from an episode of mania. He lacks insight and is unwilling to take any medication. - Locked PICU
3. Mr Vincent George is a 46-year-old man who is currently on remand in prison awaiting trial for armed robbery. He collapses on the prison wing and is found to be hypotensive, tachycardic and with marked pallor. - Emergency Department

Question 8

Not answered

Marked out of 7.00

Intellectual disability

Identify common features present in each of the following syndromes:

1. Which of the above symptoms or features are seen in Williams Syndrome? Choose TWO Options

 ❌ ❌

2. Which of the above symptoms are seen in Rett syndrome? Choose THREE Options

 ❌ ❌ ❌

3. Which of the above symptoms are seen in Lesch Nyhan syndrome? Choose TWO Options

❌ ❌

Explanation: 1) People with Williams syndrome have outgoing, engaging personalities and take interest in other people. ADHD, anxiety and phobias are common. 2) Rett syndrome occurs almost exclusively in girls. After a period of apparently normal development affected girls will lose purposeful use of their hands and make repeated hand wringing, washing or clapping motions. They tend to have small head size (microcephaly). 3) Lesch-Nyhan syndrome occurs almost exclusively in males. Affected boys experience abnormal involuntary muscle movements. Self-injury including biting and head-banging are common. (Ref: <https://ghr.nlm.nih.gov/condition/lesch-nyhan-syndrome> <https://ghr.nlm.nih.gov/condition/williams-syndrome> <https://ghr.nlm.nih.gov/condition/rett-syndrome>)

Correct Answer is :

1. Which of the above symptoms or features are seen in Williams Syndrome? Choose TWO Options - Highly sociable, ADHD

2. Which of the above symptoms are seen in Rett syndrome? Choose THREE Options - Repeating hand wringing, Microcephaly, Occurs almost exclusively in females

3. Which of the above symptoms are seen in Lesch Nyhan syndrome? Choose TWO Options - Self-biting, Occurs almost exclusively in males

Question 9

Not answered

Marked out of 4.00

Behavioural phenotypes

Which of the following genetic disorders are described in the below scenarios?

1. Amy Patterson is a 6-year-old girl who exhibits repeated hand wringing and marked behavioural disturbance.

✗

2. Jamie Walters is a 10-year-old boy who has severe learning disability but is generally described as happy and laughing much of the time.

✗

3. Kyle Peters is a 9-year-old boy who attends a mainstream primary school. His parents and teachers have noted that he tends to be rather socially awkward and shy.

✗

4. Julie Stevens is a 12-year-old girl who attends a mainstream school with some additional support. She is very outgoing with peers and interacts readily with strangers.

✗

Explanation: 1) Children with Rett's syndrome often display repetitive hand-wringing and other hand movements which often occur from early childhood and persist, with some change, into adolescence. 2) Children with Angelman's syndrome have severe intellectual impairment but are highly social with a happy demeanor and often display arm-waving and similar behaviours 3) Fragile X syndrome is characterized by social anxiety, including poor eye contact, gaze aversion, prolonged time to commence social interaction, and challenges forming friendships. Social anxiety is one of the most common features seen, with up to 75% of males in one series characterized as having excessive shyness and 50% having panic attacks. It is thought to be due to impairment of face encoding and recognition. 4) People with William's syndrome usually have mild to moderate intellectual disability with particular problems with visuospatial tasks such as drawing and fewer problems with language. They often have an outgoing personality and interact readily with strangers.

Correct Answer is :

- 1. Amy Patterson is a 6-year-old girl who exhibits repeated hand wringing and marked behavioural disturbance. - Rett's syndrome
- 2. Jamie Walters is a 10-year-old boy who has severe learning disability but is generally described as happy and laughing much of the time. -

Angelman's syndrome

3. Kyle Peters is a 9-year-old boy who attends a mainstream primary school. His parents and teachers have noted that he tends to be rather socially awkward and shy. - Fragile X syndrome

4. Julie Stevens is a 12-year-old girl who attends a mainstream school with some additional support. She is very outgoing with peers and interacts readily with strangers. - William's syndrome



Question 10

Not answered

Marked out of 1.00

Mr Jones is a 56-year-old man who is an inpatient on the surgical ward. His elective hernia repair has been delayed so he has been in hospital fasted and waiting for 24 hours. He usually drinks 6-8 cans of lager a day. He has already started to experience a headache, anxiety and mild tremor. With regards to acute alcohol withdrawal without delirium when are his symptoms likely to peak?

Select one:

- ☐ 40-50 hours
- ☐ 6-8 hours
- ☐ 72-96 hours
- ☐ 5-6 days
- ☐ 10-30 hours

Your answer is incorrect.

Most patients who drink excessive alcohol manifest a minor withdrawal syndrome, which may start as early as 6-8 hours after an abrupt reduction in alcohol intake. It can include any combination of generalized hyperactivity, anxiety, tremor, sweating, nausea, tachycardia, hypertension and mild pyrexia. These symptoms usually peak between 10 to 30 hours and subside by 40 to 50 hours. Seizures may occur in the first 12 to 48 hours and only rarely after this. Auditory and visual hallucinations may develop; these are characteristically frightening and may last for five to six days. NB: Delirium Tremens occurs usually 48-72 hours after cessation of drinking. (Reference: Alcohol Use Disorders: Diagnosis and Clinical Management of Alcohol-Related Physical Complications [Internet]. NICE Clinical Guidelines, No. 100. National Clinical Guideline Centre (UK). London: Royal College of Physicians (UK); 2010)

The correct answer is: 10-30 hours

Question 11

Not answered

Marked out of 1.00

Mr David O'Neill is a 52-year-old man who has a long history of alcohol dependence. Recently his family has noticed him "not being himself" and has brought him to the GP. Which of the following area of the brain are most likely to be damaged by alcohol?

Select one:

- ☐ Corpus callosum
- ☐ Hypothalamus
- ☐ Ventricles
- ☐ Pons
- ☐ Cerebral cortex

Your answer is incorrect.

In terms of the degree of vulnerability to chronic alcohol related damage, diencephalic structures and cerebellum are the most affected (due to thiamine disruption), followed by white matter damage, particularly affecting the genu of the corpus callosum. Working memory deficits relate to the damage sustained by the genu of the corpus callosum. While some patients exhibit cerebral cortical atrophy, this is usually a later feature, seen in those with 'alcohol-related dementia'. Ref: Sullivan EV, Harris RA, Pfefferbaum A. Alcohol's effects on brain and behavior. Alcohol Research & Health. 2010;33(1-2):127.

The correct answer is: Corpus callosum

Question 12

Not answered

Marked out of 1.00

Max is a 44-year-old man with mild alcohol dependence without significant physical sequelae. He is attending the local drug and alcohol service for his first appointment with his partner. He asks for a pharmacological intervention to reduce his craving. Which of the following treatment combinations should you offer him?

Select one:

- ☐ Naltrexone and acceptance and commitment therapy
- ☐ Buprenorphine and psychodynamic therapy
- ☐ Acamprosate and psychodynamic therapy
- ☐ Methadone and behavioural couples therapy
- ☐ Acamprosate with behavioural couples therapy

Your answer is incorrect.

NICE guidelines state that for harmful drinkers and people with mild dependence who have not responded to psychological interventions, or who request a pharmacological intervention consider offering acamprosate or naltrexone in combination with psychological intervention (individual CBT, behavioural therapy, behavioural couples therapy, or social network and environmental therapies). Acceptance and commitment therapy is not recommended in the NICE guidelines, and it is unlikely that anyone who is currently dependent on alcohol would be appropriate for psychodynamic therapy. Methadone and buprenorphine are treatments for opiate dependence rather than alcohol so are not correct. (Ref: NICE Guideline: [CG 115] (2011). Alcohol-use disorders: diagnosis, assessment and management of harmful drinking and alcohol dependence (1.3.3 Interventions for harmful drinking and mild alcohol dependence.)

<https://www.nice.org.uk/guidance/cg115/chapter/1-Guidance#interventions-for-alcohol-misuse>

The correct answer is: Acamprosate with behavioural couples therapy

Question 13

Not answered

Marked out of 1.00

Mr Colin Flood is a 43-year-old man with a long-standing history of alcohol misuse. He is brought to A&E by his friends as they are concerned about him being unwell. He is later diagnosed with Wernicke's encephalopathy. Which of the following symptom clusters is he most likely to have presented with?

Select one:

- ☐ Triad of confusion, ophthalmoplegia and cerebellar signs
- ☐ Cerebellar signs
- ☐ Cerebellar signs and confusion
- ☐ Confusion and ophthalmoplegia
- ☐ Cerebellar signs and ophthalmoplegia

Your answer is incorrect.

The most common symptom in Wernicke's is confusion, occurring in around 80% cases. Ophthalmoplegia occurs in around 30% of cases and cerebellar signs in about 23%. Only around 10% of patients with Wernicke's encephalopathy present with the classical triad of symptoms.

The correct answer is: Confusion and ophthalmoplegia

Question 14

Not answered

Marked out of 1.00

Mr. Andrews is a 28 year old man who has presented to his local Emergency Department with frank blood in his urine and pain in his abdomen. He informed staff that he has recently used illicit drugs and is concerned that this may have caused his symptoms. Which of the below illicit drugs is most likely to cause this presentation?

Select one:

- ☐ Ketamine
- ☐ Cocaine
- ☐ Heroin
- ☐ Cannabis
- ☐ MDMA

Your answer is incorrect.

Ketamine can cause severe ulcerative cystitis, which presents with abdominal pain, frank haematuria, urinary frequency, urgency and dysuria. In severe cases surgery, such as cystectomy is necessary. Up to 30% of chronic ketamine users can have urinary symptoms.

The correct answer is: Ketamine



Question 15

Not answered

Marked out of 1.00

Mr Oliver Hirsh is a 24-year-old man who has a diagnosis of paranoid schizophrenia. He was brought to A&E by his family after he presented as confused at home. In A&E he was found to be hyperthermic, tachycardic, hypertensive and showing signs of increased muscular reflexes. He managed to inform the staff that he took an overdose of Sertraline and Haloperidol earlier that day. Which of the following is the most likely diagnosis?

Select one:

- ☐ Anticholinergic toxicity
- ☐ Serotonin syndrome
- ☐ Catatonia
- ☐ Neuroleptic malignant syndrome
- ☐ Sepsis

Your answer is incorrect.

This is more likely to be Serotonin syndrome as the patient is presenting with rapid onset symptoms and hyperreflexia. Neuroleptic malignant syndrome is associated with onset of symptoms over the course of days to weeks and characterized by muscular rigidity. Anticholinergic toxicity is associated with profound visual impairment, urinary retention and marked constipation.

The correct answer is: Serotonin syndrome

Question 16

Not answered

Marked out of 1.00

Mr Keith Viner is a 38-year-old man who has presented to his GP with a 6 week history of low mood, poor motivation, diminished sleep and appetite, limited energy levels and feelings of despair about the future. He is willing to try medication but has heard that some psychiatric medications can cause sexual side-effects, which he is keen to avoid. Which of the following are least likely to cause sexual side-effects?

Select one:

- ☐ Mirtazapine
- ☐ Citalopram
- ☐ Imipramine
- ☐ Sertraline
- ☐ Venlafaxine

Your answer is incorrect.

Mirtazapine, Bupropion and Vortioxetine are the antidepressants least associated with sexual side-effects. Fewer than 10% of patients taking Mirtazapine report difficulties. Around 70% of those taking SSRIs or Venlafaxine report symptoms. TCAs are significantly associated with sexual side-effects as well.

The correct answer is: Mirtazapine

Question 17

Not answered

Marked out of 1.00

In your outpatient clinic you are seeing Miss Jackson, a 30-year-old teacher experiencing her first episode of depression. She has been prescribed 40mg citalopram for 2 months with some improvement in sleep and appetite but she continues to have negative ruminations, anxiety, suicidal thoughts, and is unable to concentrate and work. She has not had any other treatments for depression. She is on the waiting list for CBT but this will likely not be available for some months. Which ONE of the following pharmacological treatments should you offer her today?

Select one:

- ☐ Augment citalopram with lithium
- ☐ Switch to a different SSRI (selective serotonin reuptake inhibitor)
- ☐ Augment citalopram with aripiprazole
- ☐ Increase citalopram to 60mg
- ☐ Switch to a tricyclic antidepressant

Your answer is incorrect.

NICE guidelines recommend that when reviewing drug treatment for a person whose symptoms have not responded to initial pharmacological interventions consideration should be given to switching to an alternative antidepressant. Checking compliance prior to doing this is also important. NICE Guideline (CG90) 1.8.1.2 states: Consider switching to: initially a different SSRI or a better tolerated newer-generation antidepressant, subsequently an antidepressant of a different pharmacological class that may be less well tolerated, for example venlafaxine, a TCA or an MAOI. Augmentation with another agent would not be appropriate initially but may be required if other treatments are not successful. The maximum dose for citalopram is 40mg so increasing to 60mg is not advised, due to the risk of QT prolongation. (Reference: Depression in adults: recognition and management, Clinical guideline [CG90] <https://www.nice.org.uk/guidance/cg90>)

The correct answer is: Switch to a different SSRI (selective serotonin reuptake inhibitor)

Question 18

Not answered

Marked out of 1.00

Mr Jenkins is a 20-year-old man who is being reviewed in first seizure clinic. The neurologist suspects he suffers from non-epileptic attacks (or dissociative seizures) based on the history Mr Jenkins provides. Which ONE of the following patient reported seizure characteristics has the best utility in differentiating non-epileptic attacks from epilepsy?

Select one:

- ☐ Biting the inside of the mouth
- ☐ Seizures reported during sleep
- ☐ Incontinence
- ☐ Recall for the period of unresponsiveness
- ☐ Stereotyped attacks

Your answer is incorrect.

Although patients with Non-Epileptic Attacks (NEA) often report seizures in sleep, when they are captured on telemetry they are inevitably preceded by waking, which again can be recorded electrographically. Thus patient report of seizures during sleep are not necessarily helpful as discriminators and need to be examined further. The following are also not definitive discriminators: Biting the inside of the mouth occurs occasionally in NEA and commonly in epilepsy. Incontinence can occasionally occur in NEA and occurs commonly in epilepsy. Stereotyped attacks occur commonly in both NEA and epilepsy. Patients with NEA often have recall for their period of apparent unresponsiveness, whereas those with epileptic seizures do not. Therefore of the above list, this is the best answer. (Reference: Mellers JD. The approach to patients with "non-epileptic seizures". Postgraduate medical journal. 2005 Aug 1;81(958):498-504).

The correct answer is: Recall for the period of unresponsiveness

Question 19

Not answered

Marked out of 1.00

Neurosurgery for mental disorders (NMD) is an uncommon treatment modality. NICE has suggested it in certain circumstances. Which of the following disorders can be treated with NMD according to NICE guidelines?

Select one:

- ☐ OCD
- ☐ ADHD
- ☐ ASD
- ☐ Tic disorder
- ☐ Anorexia nervosa

Your answer is incorrect.

Intractable OCD can be treated via neurosurgery if other treatment modalities have failed. NMD is rarely performed with only 4 procedures carried out in 2015-16.

The correct answer is: OCD



Question 20

Not answered

Marked out of 1.00

Miss Diflorio is a 39-year-old woman who is having outpatient treatment for her stable bipolar illness. She has read recently about gender differences in mental health conditions and would like to know more about this with relevance to bipolar. Which ONE of the following bipolar spectrum conditions or comorbidities has an equal gender distribution?

Select one:

- ☐ Bipolar I
- ☐ Rapid cycling bipolar
- ☐ Comorbidity of thyroid disease
- ☐ Mixed affective states
- ☐ Bipolar II

Your answer is incorrect.

Previous studies and a review in 2010 report an almost equal gender ratio in the prevalence of bipolar disorder but the majority of studies do report an increased risk in women of bipolar II/hypomania, rapid cycling and mixed affective episodes. Comorbidity; particularly thyroid disease, migraine, obesity, and anxiety disorders occur more frequently in women than men, whilst substance use disorders are more common in men. (References: Diflorio A, Jones I. Is sex important? Gender differences in bipolar disorder. International Review of Psychiatry. 2010 Oct 1;22(5):437-52. Arnold LM. Gender differences in bipolar disorder. The Psychiatric Clinics of North America. 2003 Sep;26(3):595-620).

The correct answer is: Bipolar I

Question **21**

Not answered

Marked out of 1.00

Miss Olivia Hunt is a 33-year-old woman with a diagnosis of obsessive compulsive disorder. She has recently received an adequate trial of Sertraline 200mg daily and a course of CBT for 20 sessions. Unfortunately, this has made little difference to her symptoms. Which of the following would be the most appropriate next step in her management?

Select one:

- ☐ Add low dose Risperidone
- ☐ Change Sertraline to Clomipramine
- ☐ Change Sertraline to Mirtazapine
- ☐ Trial CBT alone
- ☐ Start Pregabalin

Your answer is incorrect.

The NICE guidelines indicate that if a patient has had an adequate trial of a combined SSRI and CBT then they should be offered a different SSRI or Clomipramine. If this is ineffective, then further intensive CBT or a low dose antipsychotic can be offered.

The correct answer is: Change Sertraline to Clomipramine



Question **22**

Not answered

Marked out of 1.00

John is 26 years old and has body dysmorphic disorder (BDD). He has been taking sertraline 200mg for the last three months and having CBT but continues to spend hours of the day looking in the mirror. He has previously tried fluoxetine at maximum dose with minimal benefit. What is the best next step in treatment?

Select one:

- ☐ Augment sertraline with buspirone
- ☐ ECT
- ☐ Switch from sertraline to venlafaxine
- ☐ Switch from sertraline to another SSRI
- ☐ Augment sertraline with mirtazapine

Your answer is incorrect.

NICE guidelines state BDD should be treated initially with CBT and then if symptoms are severe adding an SSRI. There is very limited research on head to head trials for antidepressants or for treatment resistant BDD. The Maudsley guidelines suggest that buspirone may augment the SSRI. (Ref: NICE Guidelines: Obsessive-compulsive disorder and body dysmorphic disorder: treatment. Clinical guideline [CG31] (2005). Available: <https://www.nice.org.uk/guidance/CG31/chapter/1-Guidance#steps-35-treatment-options-for-people-with-ocd-or-bdd>. Taylor, D., Paton, C. and Kapur, S. (n.d.). Anxiety Spectrum Disorders. The Maudsley prescribing guidelines in psychiatry. 12th ed. Chichester: John Wiley & Sons, pp.355).

The correct answer is: Augment sertraline with buspirone

Question **23**

Not answered

Marked out of 1.00

You are seeing a 25-year-old woman in outpatients for treatment of moderate depression. Which ONE of the following symptoms are considered a core feature of depression?

Select one:

- ☐ Talkativeness
- ☐ Pain throughout one's body
- ☐ Auditory hallucinations
- ☐ Agitation and irritability
- ☐ Panic attacks

Your answer is incorrect.

A study of outpatients with either or major depression found that across all levels of severity some symptoms were present in 60% or more of patients. The following are therefore considered core symptoms: Sad, irritable, or anxious mood, Loss of interest or pleasure, Impaired concentration and decision making, Worthlessness and inappropriate guilt, Hopelessness, Fatigue or loss of energy. Panic attacks may occur in depression with anxiety but are not a core feature. Talkativeness would suggest hypomania or mania. Auditory hallucinations can occur in psychotic depression but are not a core feature of depression. All over body pain can occur in depression, especially in particular ethnic groups but is not an agreed core symptom according to current criteria. (Ref: Rakofsky JJ, Schettler PJ, Kinkead BL, Frank E, Judd LL, Kupfer DJ, Rush AJ, Thase ME, Yonkers KA, Rapaport MH. The prevalence and severity of depressive symptoms along the spectrum of unipolar depressive disorders: a post hoc analysis. The Journal of clinical psychiatry. 2013 Nov;74(11):1084-91)

The correct answer is: Agitation and irritability

Question **24**

Not answered

Marked out of 1.00

Mr Fox is a 60-year-old man who is taking lithium for stable bipolar disorder. He presents to A&E with confusion and falls in the context of a recent chest infection. Which of the following features would be NOT be consistent with acute lithium toxicity?

Select one:

- ☐ Fever
- ☐ Bradycardia
- ☐ Nystagmus
- ☐ Coarse tremor
- ☐ Prolonged QTc on ECG

Your answer is incorrect.

Nystagmus, ataxia, sluggishness, agitation, fasciculations and coarse tremor are neurological features of lithium toxicity. Reported cardiac effects are QTc prolongation and bradycardia. Fever is not seen in lithium toxicity and would suggest sepsis or the need to consider neuroleptic malignant syndrome, depending on the other symptoms. (Ref: Perrone J, Chatterjee P. Lithium poisoning. <http://www.uptodate.com>. Wolters Kluwer Health. 2015:1-3)

The correct answer is: Fever

Question **25**

Not answered

Marked out of 1.00

Mrs Carol Anwar is a 40-year-old woman who has a diagnosis of HIV and a history of having difficulty taking her antiretrovirals regularly. She has been admitted to A&E in a confused and feverish state. Her MRI scan shows multiple ring-enhancing lesions. Which of the following is the most likely diagnosis?

Select one:

- ☐ Herpes simplex encephalitis
- ☐ Tuberculosis
- ☐ Toxoplasmosis
- ☐ Cryptococcus neoformans
- ☐ Pneumococcal meningitis

Your answer is incorrect.

Multiple ring enhancing lesions on MRI in patients with a diagnosis of HIV tend to indicate either CNS toxoplasmosis or CNS lymphoma. The two are generally distinguished by CT scan.

The correct answer is: Toxoplasmosis



Question **26**

Not answered

Marked out of 1.00

Mrs Tara Neville is a 32-year-old woman with a diagnosis of schizophrenia. She has been well for 3 years and she and her husband are planning on having children. She is wondering what the risk of them developing schizophrenia would be. Which of the following would be the most appropriate answer?

Select one:

- ☐ 5-9%
- ☐ 10-15%
- ☐ 21-24%
- ☐ 16-20%
- ☐ 25-30%

Your answer is incorrect.

Studies indicate that children have a risk of around 9-14% of developing schizophrenia if one of their parents has the diagnosis.

The correct answer is: 10-15%



Question 27

Not answered

Marked out of 1.00

A young male patient on the acute psychiatric ward is known to have non-epileptic attacks. After a ward round he is observed in the common area on the sofa with jerky movements and appearing to have an attack. Which ONE the following features are more likely to suggest that this is an epileptic rather than a non-epileptic attack?

Select one:

- ☐ Impaired ability to recall the attack
- ☐ Side to side head movements during attack
- ☐ Evidence of visual fixation during the attack
- ☐ Downgoing plantar reflexes after the attack
- ☐ Resistance to eye opening during the attack

Your answer is incorrect.

Side to side head movements and resistance to eye opening are more common in Non-Epileptic Attacks. Neurological testing during a Non-Epileptic Attack will find that visual fixation is present (whereas it is not in an epileptic attack). Plantars are often up-going after an epileptic attack and should be normal after a non-Epileptic Attack. Inability to recall the attack and post-ictal confusion is suggestive of an epileptic attack. (Ref: Mellers JD. The approach to patients with "non-epileptic seizures". Postgraduate medical journal. 2005 Aug 1;81(958):498-504)

The correct answer is: Impaired ability to recall the attack

Question **28**

Not answered

Marked out of 1.00

Which ONE mood-stabilizing medication used in bipolar is associated with the greatest reduction in risk of attempted and completed suicide?

Select one:

- ☐ Valproic acid
- ☐ Lithium
- ☐ Sodium valproate
- ☐ Gabapentin
- ☐ Carbamazepine

Your answer is incorrect.

Around 15% of people with bipolar complete suicide. Meta-analyses of clinical trials have demonstrated that lithium reduced the risk of both attempted and completed suicide. Two large database studies have shown that patients taking lithium were less likely to complete suicide than those treated with valproate, valproic acid or other mood stabilizing drugs such as gabapentin or carbamazepine. (Ref: Taylor, D., Paton, C. and Kapur, S. (n.d.). Chapter 3: Bipolar affective disorder. The Maudsley prescribing guidelines in psychiatry. 12th ed. Chichester: John Wiley & Sons, pp.190)

The correct answer is: Lithium

Question 29

Not answered

Marked out of 1.00

Numerous risk factors have been identified for the development of schizophrenia. Which of the following is most likely to place an individual at risk of developing schizophrenia?

Select one:

- ☐ Having 3 or more siblings
- ☐ Alcohol binges during adolescence
- ☐ Tobacco use
- ☐ Migration
- ☐ Left-handedness

Your answer is incorrect.

Migration has been consistently associated with the development of schizophrenia.

The correct answer is: Migration



Question 30

Not answered

Marked out of 1.00

Mrs Lisa Lodge is a 34-year-old woman who is 2 months post-partum. Her health visitor notices that she is presenting as low in mood struggling to bond with her baby. She administers the Edinburgh Post-natal Depression Scale. Which of the following is true regarding this tool?

Select one:

- ☐ It only measures depressive symptoms
- ☐ It recommended that it is used at 4 months post-partum
- ☐ It has only been validated in the post-partum period
- ☐ It can be used for diagnosis
- ☐ A high score predicts suicidality

Your answer is incorrect.

A high score on the EPNDs predicts suicidality, particularly if the patient answers positively to question 10: "the thought of harming myself has occurred to me". It has been conceptualised as measuring depressive, anxiety and anhedonia symptoms and has been validated for use in pregnancy and even in general population. It is a screening tool and higher scores can indicate a wide range of possible diagnoses. It is generally used at 6-8 weeks post-partum.

The correct answer is: A high score predicts suicidality

Question **31**

Not answered

Marked out of 1.00

Many mental health conditions are more prevalent amongst males or females. Which of the following has equal prevalence for males and females?

Select one:

- ☐ Unipolar depression
- ☐ Social phobia
- ☐ Generalized anxiety
- ☐ PTSD
- ☐ Hypochondriasis

Your answer is incorrect.

Social phobia has equal prevalence for males and females in community samples. In clinic samples males have typically been over-represented.

The correct answer is: Social phobia



Question **32**

Not answered

Marked out of 1.00

Jackie is a 24-year-old woman who is recovering from her first episode of psychosis where she was treated with risperidone. She has gained more than 10kg in the last 3 months since starting treatment and now has raised lipids. She is wondering if she can change to another antipsychotic. Which ONE of the following antipsychotics has the least potential to cause metabolic syndrome?

Select one:

- ☐ Paliperidone
- ☐ Aripiprazole
- ☐ Olanzapine
- ☐ Chlorpromazine
- ☐ Quetiapine

Your answer is incorrect.

Clozapine and olanzapine all have a high risk of weight gain and metabolic syndrome. Quetiapine, Paliperidone and Chlorpromazine have a moderate risk. Aripiprazole has low risk for weight gain and metabolic syndrome. (Ref: Taylor, D., Paton, C. and Kapur, S. (n.d.). Antipsychotic induced weight gain. The Maudsley prescribing guidelines in psychiatry. 12th ed. Chichester: John Wiley & Sons, pp.90)

The correct answer is: Aripiprazole

Question 33

Not answered

Marked out of 1.00

Mr Ross Giles is a 32-year-old man with a diagnosis of treatment resistant schizophrenia who has recently started taking Clozapine. He reports to his care co-ordinator that he has been feeling generally unwell and they arrange for him to have some blood tests. His neutrophils are $5 \times 10^9/L$ and his CRP is 60mg/L. You are the doctor on duty for the team. Which would be the next most appropriate investigation?

Select one:

- ☐ Abdominal x-ray
- ☐ ECG
- ☐ Chest x-ray
- ☐ Urinalysis
- ☐ Blood cultures

Your answer is incorrect.

General malaise and a high CRP in the context of starting Clozapine raise the possibility of myocarditis which is a serious but rare side-effect. This is particularly the case in the absence of any evidence of a specific source of infection or other inflammation. Myocarditis must be ruled out in the first instance as it can be fatal if not identified and treated promptly.

The correct answer is: ECG

Question **34**

Not answered

Marked out of 1.00

Jane is a 15-year-old girl currently admitted under the mental health act to an adolescent unit. She is having her first episode of mania with psychotic features, having previously had a history of depression and been prescribed fluoxetine. Which ONE of the following would be appropriate as a single agent in treatment of Jane's mania?

Select one:

- ☐ Haloperidol
- ☐ Sodium Valproate
- ☐ Carbamazepine
- ☐ Lithium
- ☐ Risperidone

Your answer is incorrect.

Recommended first line treatments for acute mania in children and adolescents include atypical antipsychotics such as aripiprazole, olanzapine, quetiapine and risperidone. Lithium can be used but would not be appropriate for Jane as a monotherapy as she has psychotic symptoms. Carbamazepine is not recommended. Sodium Valproate has not shown efficacy in a small RCT for adolescent mania. (Reference: Taylor, D., Paton, C. and Kapur, S. (n.d.). Bipolar illness in children and adolescents. The Maudsley prescribing guidelines in psychiatry. 12th ed. Chichester: John Wiley & Sons, pp.364-365).

The correct answer is: Risperidone

Question 35

Not answered

Marked out of 1.00

James is a 17-year-old boy who was diagnosed with ADHD at the age of 8. He is seeing you in CAMHS and has asked about the neurobiological basis of his illness. He is aware the fronto-striatal network is implicated. Which brain region does NOT form part of the fronto-striatal network?

Select one:

- ☐ Dorsal anterior cingulate cortex
- ☐ Caudate nucleus
- ☐ Putamen
- ☐ Lateral pre-frontal cortex
- ☐ Cerebellum

Your answer is incorrect.

Data from neuroimaging, neuropsychology, neurochemistry and genetics consistently implicate the fronto-striatal network as a major contributor to the pathophysiology of ADHD. The network involves the lateral prefrontal cortex, the dorsal anterior cingulate cortex, the caudate nucleus and putamen. The cerebellum is not part of the network. (Ref: Emond V, Joyal C, Poissant H. Structural and functional neuroanatomy of attention-deficit hyperactivity disorder (ADHD). *L'encéphale*. 2009 Apr;35(2):107-14).

The correct answer is: Cerebellum

Question **36**

Not answered

Marked out of 1.00

Mr Neil Slater is a 16-year-old boy with a diagnosis of autistic spectrum disorder. He attends a special school and is soon to go to college. What is the likelihood that he would also have a diagnosis of epilepsy?

Select one:

- ☐ 12%
- ☐ 40%
- ☐ 22%
- ☐ 33%
- ☐ 5%

Your answer is incorrect.

A recent study suggested that 22% of people with an autism spectrum disorder develop epilepsy by the age of 21 years. This is in keeping with previous studies. (Ref: Bolton PF, Carcani-Rathwell I, Hutton J, Goode S, Howlin P, Rutter M. Epilepsy in autism: features and correlates. The British Journal of Psychiatry. 2011 Apr;198(4):289-94.)

The correct answer is: 22%



Question 37

Not answered

Marked out of 1.00

Miss Joan Stafford is a 15-year-old girl who has been suffering from persistent low mood, reduced interest in activities, irritability, poor sleep and appetite, and diminishing school performance. She has had a trial of CBT which was not effective. Her parents are not willing for her to have any medications. Which of the following would be the most appropriate next step for her treatment?

Select one:

- ☐ Repeat trial of CBT
- ☐ Systemic family therapy
- ☐ Individual child psychotherapy
- ☐ Interpersonal therapy
- ☐ Non-directive supportive counselling

Your answer is incorrect.

NICE guidelines for treatment of depression in children and adolescents' states that if a trial of an initial psychological intervention is unsuccessful, an alternative, such of CBT, interpersonal therapy or family therapy should be considered. Fluoxetine can also be considered if the patient or carers are accepting of this. Repeating a therapy that has been ineffective would not be helpful and counselling is unlikely to be successful if a formal psychotherapy has not worked. Individual psychotherapy and systemic family therapy are generally offered after two trials of psychotherapy has been unsuccessful.

The correct answer is: Interpersonal therapy

Question **38**

Not answered

Marked out of 1.00

Oliver is a 10-year-old boy who sustained a head injury falling from his skateboard 2 months ago. At the time he lost consciousness for less than 1 minute and had a GCS of 14 when he was later seen in A&E. He had some vomiting which settled within 24 hours of the injury. You are now seeing him 2 months later in CAMHS and his mother is wondering if there is a link between the head injury and his current complaint. Which ONE of the following neuropsychiatric syndromes or symptoms is NOT associated with mild Traumatic Brain Injury (mTBI) in children?

Select one:

- ☐ ADHD
- ☐ Conduct disorder
- ☐ Depression
- ☐ Mania
- ☐ Anxiety

Your answer is incorrect.

A systematic review found consistent associations between mTBI and ADHD, conduct disorder, depression and anxiety. However the review did suggest that the lack of pre-injury reports make firm conclusions difficult. Mania has not been reported. (Ref: Emery CA, Barlow KM, Brooks BL, Max JE, Villavicencio-Requis A, Gnanakumar V, Robertson HL, Schneider K, Yeates KO. A systematic review of psychiatric, psychological, and behavioural outcomes following mild traumatic brain injury in children and adolescents. The Canadian Journal of Psychiatry. 2016 May;61(5):259-69).

The correct answer is: Mania

Question 39

Not answered

Marked out of 1.00

Andrew Davies is a 9-year-old boy who has a diagnosis of autism spectrum disorder. His mother has noticed that sometimes he can comment on individual aspects of an object but not seem to see it as a 'whole'. She gives the example of him being able to talk about wheels on a toy car but not being able to identify its overall shape. Which of the following best describes this phenomenon?

Select one:

- ☐ Sensory processing dysfunction
- ☐ Central coherence theory
- ☐ Theory of mind
- ☐ Attribution bias
- ☐ Hyper-systemisation

Your answer is incorrect.

Central coherence theory underlies the central disturbance seen in people with autistic spectrum disorders. It is considered as a specific perceptual-cognitive style. It is loosely described as a limited ability to understand context or to "see the big picture".

The correct answer is: Central coherence theory



Question **40**

Not answered

Marked out of 1.00

Forensic psychiatrists, as with other specialists within mental health, consider various factors when formulating the difficulties of their patients, including organic ones. Damage to which of the following areas of the brain would a forensic psychiatrist be most interested in?

Select one:

- ☐ Amygdala
- ☐ Cerebellar vermis
- ☐ Hippocampus
- ☐ Prefrontal cortex
- ☐ Hypothalamus

Your answer is incorrect.

Damage to the frontal lobe is associated with a variety of symptoms relating to executive functioning and personality. The prefrontal cortex is particularly associated with impulse control which is potentially relevant to forensic psychiatrists.

The correct answer is: Prefrontal cortex



Question **41**

Not answered

Marked out of 1.00

Mr James Bulwell is a 32-year-old man who has been serving a life sentence for murder. He has developed a psychotic illness during his time in prison. He is presenting as chaotic and requires treatment. Which of the following would be the most appropriate venue for his treatment?

Select one:

- ☐ Prison hospital wing
- ☐ General population prison wing
- ☐ General psychiatric ward
- ☐ Psychiatric intensive care unit
- ☐ Medium secure psychiatric ward

Your answer is incorrect.

It would be most appropriate for him to be transferred to a medium secure forensic ward, although he may require high secure depending upon his presentation and risk history. It is unlikely that he could be managed in the prison if he is chaotic as the ability to deliver treatment to an incapacitous patient is limited. A general psychiatric ward would be inappropriate due the degree of risk. As would a PICU as they are usually only considered to be low secure.

The correct answer is: Medium secure psychiatric ward

Question 42

Not answered

Marked out of 1.00

Confidentiality is an important concept in psychiatry, and general medical, practice. Which of the following cases refers to matters around confidentiality?

Select one:

- ☐ McKenzie
- ☐ Willowbrook School
- ☐ Tuskegee
- ☐ Gillick
- ☐ Tarasoff

Your answer is incorrect.

The Tarasoff case relates to confidentiality. Tatiana Tarasoff was murdered by a classmate at college who had previously indicated to a psychologist that he had a plan to kill her. The psychologist had not disclosed this information to the authorities or Tatiana Tarasoff. It led to court judgements in 1974 and 1976 that health professionals had a duty to breach confidentiality in cases when they are aware of a realistic threat to others.

The correct answer is: Tarasoff



Question **43**

Not answered

Marked out of 1.00

Prison populations are well recognized as having high rates of mental health disorders. Which of the following statements about mental health in prisons is true?

Select one:

- ☐ Personality disorder is the most common diagnosis
- ☐ Male prisoners are more likely to suffer from depression and anxiety than female prisoners
- ☐ Most non-violent offenders are not dependent on alcohol
- ☐ Male prisoners carry out deliberate self-harm more often than female prisoners
- ☐ The prevalence of schizophrenia in the prison population is similar to the general population.

Your answer is incorrect.

Personality disorder, particularly antisocial PD is the most common diagnosis among prison populations. There is significant overlap between the diagnostic criteria for this and the factors that would lead to incarceration.

The correct answer is: Personality disorder is the most common diagnosis



Question **44**

Not answered

Marked out of 1.00

Mr Barlow is a 35-year-old man who has just been convicted of GBH, common assault and theft. He will be sentenced to 9 years in prison. He has asked to see the prison psychiatrist following his initial intake review. Regarding the male prison population as a whole, what is the most prevalent mental disorder?

Select one:

- ☐ Post Traumatic Stress Disorder
- ☐ Depression
- ☐ Antisocial personality disorder
- ☐ Alcohol misuse or dependence
- ☐ Psychotic illness

Your answer is incorrect.

The estimated prevalence of antisocial PD is 47%, of alcohol misuse or dependence 18-30%, posttraumatic stress disorder 4-21%, depression 10%, and psychosis 4%. (Source: Ginn, S. Dealing with mental disorder in prisoners BMJ 2012; 345 :e7280)

The correct answer is: Antisocial personality disorder



Question 45

Not answered

Marked out of 1.00

Mr. Young is a 28-year-old man with a diagnosis of schizophrenia who is currently under the mental health act in a medium secure unit. He is repeatedly presenting as 'aggressive'. Which of the following would be the most effective method to manage his behaviour?

Select one:

- ☐ Isolation
- ☐ Interpersonal therapy
- ☐ Physical restraint
- ☐ Antipsychotics
- ☐ CBT

Your answer is incorrect.

Effective treatment of his underlying psychotic illness would be the most effective way to reduce aggressive behaviour. There is some evidence that clozapine may be superior to other antipsychotics in terms of managing aggression.

The correct answer is: Antipsychotics



Question 46

Not answered

Marked out of 1.00

Mental health problems and violent offending are often highlighted by the media and described by the public as a concern. Which of the following statements about mental health and violence is true?

Select one:

- ☐ 70% of mental health patients who commit crimes are dependent on alcohol
- ☐ People with a mental health problem are more likely to be the victim of violent crime than be the perpetrator
- ☐ Patients with a substance dependence disorder are as likely to be assaulted as those without
- ☐ If a mental health problem is associated with violence, effective treatment does not influence the risk to others
- ☐ People with a mental health problem are more likely to commit violent crimes than those without

Your answer is incorrect.

People with mental health problems are far more likely to be the victims of violent crime than perpetrators. Schizophrenia and other psychoses can be associated with increased rates of violence towards others, but this is still less likely than them being victims.

The correct answer is: People with a mental health problem are more likely to be the victim of violent crime than be the perpetrator



Question **47**

Not answered

Marked out of 1.00

Mental health problems are more common in offending populations. Which of the following is the most common psychiatric disorder among homicide offenders?

Select one:

- ☐ Personality disorders
- ☐ Organic disorders
- ☐ Depression
- ☐ Anxiety disorders
- ☐ Schizophrenia

Your answer is incorrect.

9% of people convicted of homicide had a diagnosis of personality disorder. The figures for other disorders were: schizophrenia – 5%; affective disorder – 7%; alcohol dependence – 7%; drug dependence – 6%.

The correct answer is: Personality disorders



Question 48

Not answered

Marked out of 1.00

Mr Elsworth is a 39-year-old man who is an inmate in a category B prison following a sexual offence. He undergoes a risk assessment, accounting for historical and psychological factors that would influence the probability of him offending again. Which of the following factors would increase the risk of sexual reoffending the most?

Select one:

- ☐ Intra-familial incest
- ☐ Possession of child pornography
- ☐ Dissocial personality disorder and Paedophilic paraphilia
- ☐ History of depression
- ☐ Sexual assault with penetration of a stranger

Your answer is incorrect.

Offenders who demonstrate a persistent lack of empathy and guilt, as in dissocial personality disorder, and who demonstrate evidence of sexual deviance, such as paedophilia, are most likely to sexually reoffend. Those who sexually assault strangers, and particularly those who commit rape, are likely to reoffend as well, but show more general criminal behaviour and will commit other violent offences.

The correct answer is: Dissocial personality disorder and Paedophilic paraphilia

Question **49**

Not answered

Marked out of 1.00

Ganser syndrome is a rare mental disorder, which presents with a particular set of symptoms. Which of the following are characteristic of Ganser syndrome?

Select one:

- ☐ Agitation
- ☐ Somnolence
- ☐ Visual hallucinations
- ☐ Tremor
- ☐ Mutism

Your answer is incorrect.

Ganser syndrome is a factitious disorder characterised by symptoms of approximate answers to questions, somatic conversion symptoms, clouding of consciousness and perceptual disturbances, including visual hallucinations. It was first described in male prison populations but is now recognised in other settings. It can be challenging to differentiate it from malingering in some settings.

The correct answer is: Visual hallucinations



Question 50

Not answered

Marked out of 1.00

Mr Green is a 35 year old man who is due to be released from prison. Whilst an inmate, serving a sentence for grievous bodily harm and theft he was treated in the healthcare wing for depression and was also diagnosed with antisocial personality disorder. Which of the following factors would be associated with an increased risk of re-offending?

Select one:

- ☐ Having served a 10 year sentence
- ☐ Returning to his parent's home immediately following release
- ☐ Having an index offence which was violent in nature
- ☐ Suffering from depression
- ☐ Returning to amphetamine use following release

Your answer is incorrect.

Dynamic factors, which are associated with reoffending, include drug misuse, alcohol misuse, impulsivity, relationship with anti-social peers, lack of family relationships, lack of employment, having unsuitable accommodation. Static factors which are associated with a reduced risk of reoffending include: first time in custody, employment in the 12 months before custody, being worried about having spent time in prison, being older, longer sentences (greater than 1 year). Therefore returning to amphetamine use following discharge would be associated with an increased risk of reoffending. (Ref:https://www.gov.uk/government/uploads/system/uploads/attachment_data/file/243718/evidence-reduce-reoffending.pdf)

The correct answer is: Returning to amphetamine use following release

Question **51**

Not answered

Marked out of 1.00

Eva has a diagnosis of high functioning autism. She has a particular fascination with patterns in music and is able to perceive and describe complex reoccurring patterns. What description can be given to this particular talent?

Select one:

- ☐ Extreme male brain
- ☐ Selective abstraction
- ☐ Operant conditioning
- ☐ Positive reinforcement
- ☐ Hypersystemization

Your answer is incorrect.

Systematizing is the drive to analyse or construct systems and recognising repeating patterns in stimuli. This is a common feature of those with autistic spectrum disorders (see Philos Trans R Soc Lond B Biol Sci 2009; 364 (1522): 1377-1383)

The correct answer is: Hypersystemization



Question 52

Not answered

Marked out of 1.00

Mr David McDermott is a 40-year-old man with a diagnosis of DiGeorge syndrome. He has recently been referred to his local psychiatric clinic due to concerns about his mental health. Which of the following psychiatric disorders is most strongly associated with DiGeorge syndrome?

Select one:

- ☐ Schizophrenia
- ☐ Depression
- ☐ Agoraphobia
- ☐ Body dysmorphic disorder
- ☐ Bipolar affective disorder

Your answer is incorrect.

Schizophrenia is most strongly associated, with up to 30% of those with DiGeorge syndrome developing a psychotic illness.

The correct answer is: Schizophrenia



Question 53

Not answered

Marked out of 1.00

In your CAMHS clinic you are seeing Skye, a 3-year-old child with developmental delay. Her mother reports she had hypocalcaemia soon after she was born. Which ONE of the following syndromes are associated with neonatal hypocalcaemia?

Select one:

- ☐ Down's syndrome
- ☐ Rubinstein-Taybi syndrome
- ☐ Angelman syndrome
- ☐ Fragile X syndrome
- ☐ DiGeorge syndrome

Your answer is incorrect.

In DiGeorge syndrome neonatal hypocalcaemia can result from hypoparathyroidism secondary to parathyroid aplasia or hypoplasia. The other disorders listed are not associated with neonatal hypocalcaemia. (References: Carpenter et al, 2017. Etiology of neonatal hypocalcaemia. <https://www.uptodate.com/contents/etiology-of-hypocalcemia-in-infants-and-children>)

The correct answer is: DiGeorge syndrome

Question 54

Not answered

Marked out of 1.00

John is a young man with fragile X whom you are seeing in the learning disabilities service. He has been assessed by the speech and language therapist and you are reading the report. Which of the following speech or cognitive problems would NOT be usually observed in Fragile X syndrome?

Select one:

- ☐ Perseverative language use
- ☐ Tangential language use in conversation
- ☐ Delayed echolalia
- ☐ Impaired syntax
- ☐ Weak auditory processing skills

Your answer is incorrect.

The types of atypical language most often seen in Fragile X syndrome in conversational interactions include tangential language, perseverative language, repetitive speech and tendency toward delayed echolalia. Short bursts of speech with disruptions in the flow, including repetitions of sounds, words, and phrases are often heard. Fragile X speech has been described as "cluttered". Syntax (arrangement of words and phrases) is usually appropriate for mental age; a high receptive vocabulary score is usually seen, and auditory sensory and processing skills are weak. (Ref: Cornish K, Sudhalter V, Turk J. Attention and language in fragile X. Mental retardation and developmental disabilities research reviews. 2004 Feb;10(1):11-6).

The correct answer is: Impaired syntax

Question 55

Not answered

Marked out of 1.00

Mrs Hildebrand has brought her daughter with Down Syndrome to see you in the CAMHS clinic. She is wanting to know her daughter's prognosis in terms of intelligence and potential functioning. Which ONE of the following Down Syndrome variants may have the best potential to attain a higher IQ than the others?

Select one:

- ☐ Translocation to chromosome 22
- ☐ Full trisomy 21
- ☐ Mosaicism
- ☐ Robertsonian translocation

Your answer is incorrect.

Individuals with mosaicism have variable proportions of normal and abnormal cells. One study has found that the levels of trisomy cells in buccal mucosa is associated with IQ. Those with a fewer number of trisomy cells had a higher IQ. Therefore, those with mosaicism may (but will not necessarily) have the potential to approach normal IQ. Trisomy 13 is Patau syndrome and not Down Syndrome. (References: Papavassiliou P, York TP, Gursoy N, Hill G, Nicely LV, Sundaram U, McClain A, Aggen SH, Eaves L, Riley B, Jackson-Cook C. The phenotype of persons having mosaicism for trisomy 21/Down syndrome reflects the percentage of trisomic cells present in different tissues. American journal of medical genetics Part A. 2009 Apr;149(4):573-83)

The correct answer is: Mosaicism

Question 56

Not answered

Marked out of 1.00

You are seeing Mr Basu who is 22 years old and has Down's Syndrome. His mother reports great difficulty in arranging for him to have cataract surgery. Both mother and son speak Bengali. Based on research which of the following has NOT been reported as a barrier for people with intellectual disabilities to access healthcare services?

Select one:

- ☐ Issues with service eligibility
- ☐ Difficulty in accessing interpreters
- ☐ Administrative errors
- ☐ Failure of services to make reasonable adjustments
- ☐ Negative staff attitudes and behaviour

Your answer is incorrect.

A recent qualitative study has looked at the barriers patients with intellectual disabilities face in accessing health care. Interviews were conducted with patient carers. They reported accounts of negative staff attitudes and behaviour, failure of services to make reasonable adjustments. Other barriers included problems with communication, and accessing services because of lack of knowledge of local services and service eligibility issues; lack of support and involvement of carers; and language problems in participants from minority ethnic groups. (Ref: Ali A, Scior K, Ratti V, Strydom A, King M, Hassiotis A. Discrimination and other barriers to accessing health care: perspectives of patients with mild and moderate intellectual disability and their carers. PloS one. 2013 Aug 12;8(8):e70855.)

The correct answer is: Administrative errors

Question **57**

Not answered

Marked out of 1.00

Mrs Claremont is a 30-year-old woman who is currently 10 weeks pregnant. She is wondering about the probability of a woman of her age having a child with Down's Syndrome. What is the probability of a woman of the age of 30 having a child with Down's Syndrome?

Select one:

- ☐ 1/45
- ☐ 1/450
- ☐ 1/90
- ☐ 1/900
- ☐ 1/1200

Your answer is incorrect.

A 30-year-old woman has a roughly 1/900 probability of having a baby with Down's syndrome. For a 25 year old, this is about 1/1200, by 35, the risk has increased to 1/350, by age 40 it is 1/100 and by 49 it is 1/10.

The correct answer is: 1/900



Question 58

Not answered

Marked out of 1.00

Mr O'Mahony has had Parkinson's disease for 2 years and has now been diagnosed with Parkinson's dementia. He has recently started experiencing distressing visual hallucinations after having seen his neurologist last week. What would be the first line management strategy for new psychotic symptoms in someone with Parkinson's dementia?

Select one:

- ☐ Commence haloperidol 0.5mg BD
- ☐ Commence lithium 200mg nocte
- ☐ Review Parkinson's medications and consider reducing in consultation with neurologist
- ☐ Commence donepezil 5mg nocte
- ☐ Commence quetiapine 12.5mg BD

Your answer is incorrect.

The NICE guidelines for Parkinson's disease state that the first course of action in new hallucinations is to reduce the dosage of any Parkinson's disease medicines that might have triggered hallucinations or delusions, taking into account the severity of symptoms and possible withdrawal effects. Seek advice from a healthcare professional with specialist expertise in Parkinson's disease before modifying therapy. Haloperidol is contraindicated in Parkinson's disease for its potential to worsen Parkinsonism and cause life threatening extra pyramidal side effects. Lithium is not indicated. Quetiapine may be considered if the symptoms are distressing and do not respond to other treatments. However, if Mr O'Mahony is not yet on a cholinesterase inhibitor this would be the best medication to start prior to adding antipsychotics. (Ref: Parkinson's disease in adults. NICE guideline [NG71] Published date: July 2017. <https://www.nice.org.uk/guidance/ng71/chapter/Recommendations#pharmacological-management-of-non-motor-symptoms>)

The correct answer is: Review Parkinson's medications and consider reducing in consultation with neurologist

Question 59

Not answered

Marked out of 1.00

Mrs Brenda Yung is a 82 year old woman with a diagnosis of mild Alzheimer' disease who wishes to remain in her home as long as possible. Her family is keen for her to move to a residential home. They are querying her capacity to make a decision around this. Which of the following is not true regarding mental capacity?

Select one:

- ☐ It can be enhanced by repeated discussions
- ☐ It can only be assessed by a psychiatrist
- ☐ It applies to general decisions about every day issues
- ☐ It can vary and be present at sometimes and not at others
- ☐ It may be impaired for one decision but not another

Your answer is incorrect.

Mental capacity does not have to be assessed by a psychiatrist. A wide variety of healthcare and legal professionals should be able to assess capacity, particularly with reference to their specific professional roles. Psychiatrists can be asked to assist if the decision is difficult or contentious.

The correct answer is: It can only be assessed by a psychiatrist

Question 60

Not answered

Marked out of 1.00

What is the first line medication for a patient newly diagnosed with mild-moderate Lewy Body Dementia?

Select one:

- ☐ Galantamine
- ☐ Memantine
- ☐ Quetiapine
- ☐ Trazadone
- ☐ Rivastigmine

Your answer is incorrect.

The NICE guidelines state the following: Offer donepezil or rivastigmine to people with mild to moderate dementia with Lewy Bodies. Only consider galantamine for the above patient group if donepezil or rivastigmine are not tolerated. Consider memantine for people with dementia with Lewy bodies if AChE inhibitors are not tolerated or are contraindicated. Trazadone is an antidepressant and not a first line treatment for dementia. Quetiapine is not indicated as there are no psychotic symptoms reported, and even if there were, an AChE inhibitor would be the first line of treatment. (Reference: Dementia: assessment, management and support for people living with dementia and their carers. NICE guideline [NG97] Published date: June 2018. <https://www.nice.org.uk/guidance/ng97/chapter/Recommendations#managing-non-cognitive-symptoms>)

The correct answer is: Rivastigmine

Question 61

Not answered

Marked out of 1.00

Which of the following is a core feature of dementia with Lewy bodies?

Select one:

- ☐ Persecutory delusions
- ☐ Auditory hallucinations
- ☐ Depression
- ☐ Visual hallucinations
- ☐ Stable level of alertness

Your answer is incorrect.

In addition to cognitive decline (dementia), a patient with probable Dementia with Lewy Bodies (DLB) must also have at least two "core clinical features" which includes: cognitive fluctuations, visual hallucinations, rapid eye movement (REM) sleep behaviour disorder, and parkinsonism. Other features such as antipsychotic sensitivity, falls, hallucinations in other modalities, delusions, anxiety and depression may be supportive but are not core features. (Ref: Farlow RF, DeKosky ST, Eichler AF. (2018) Clinical features and diagnosis of dementia with Lewy bodies. <https://www.uptodate.com/contents/clinical-features-and-diagnosis-of-dementia-with-lewy-bodies>)

The correct answer is: Visual hallucinations

Question 62

Not answered

Marked out of 1.00

Mabel is 82 years old and lives alone. She has recently been diagnosed with late onset schizophrenia (or paraphrenia). Which ONE of the following symptoms are more commonly found in late onset schizophrenia when compared to adult onset schizophrenia?

Select one:

- ☐ Formal thought disorder
- ☐ Partition delusions
- ☐ Thought insertion
- ☐ Blunting of affect
- ☐ Catatonia

Your answer is incorrect.

A few studies have compared early onset and late onset schizophrenia (previously known as paraphrenia) with adult onset schizophrenia identifying significant differences in phenomenology and symptomatology. Persecutory delusions and partition delusions are more common in late onset schizophrenia. Catatonia, thought insertion and withdrawal are more common in young onset as are negative symptoms such as blunting of affect. Formal thought disorder is also more common in younger age groups. (Ref: Howard R, Castle D, Wessely S, Murray R. A comparative study of 470 cases of early-onset and late-onset schizophrenia. The British journal of psychiatry. 1993 Sep;163(3):352-7)

The correct answer is: Partition delusions

Question 63

Not answered

Marked out of 1.00

Miss Laura Brookes is a 55-year-old woman who has been diagnosed with REM sleep disorder due to excessive movements during sleep. Which of the following is this diagnosis associated with?

Select one:

- ☐ OCD
- ☐ Alzheimer's disease
- ☐ Dementia with Lewy Bodies
- ☐ Frontal lobe dementia
- ☐ Schizophrenia

Your answer is incorrect.

REM sleep behavioural disorder is characterized by patients acting out their dreams during REM sleep and involves the loss of normal muscle tone during this phase of sleep. It is strongly associated with later development of synucleopathies, such as Parkinson's disease and Lewy Body Dementia.

The correct answer is: Dementia with Lewy Bodies



Question **64**

Not answered

Marked out of 1.00

Depression is commonly co-morbid with dementia but can be more difficult to identify in patients with clinically significant cognitive impairment. Which of the followings are most useful for detecting depression in people with dementia?

Select one:

- ☐ Beck's Depression Inventory
- ☐ MADRAS
- ☐ Hamilton
- ☐ Geriatric depression scale
- ☐ Cornell scale

Your answer is incorrect.

The Cornell Scale for Depression in Dementia is designed for the assessment of depression in older people with dementia who can at least communicate basic needs. It has been tested for reliability, sensitivity and validity on patients in community, hospital and care home settings. Scores are determined by a combination of prior observation and two interviews: 20 minutes with the carer and 10 minutes with the patient.

The correct answer is: Cornell scale

Question 65

Not answered

Marked out of 1.00

You have just diagnosed Mr Jackson with mixed Alzheimer's and vascular dementia in the memory clinic. His daughter phones you afterwards to inquire about risk factors she might address to reduce her chance of developing dementia. Which ONE of the following statements is INCORRECT with regards to associations with dementia?

Select one:

- ☐ Obesity associated with increased risk for Alzheimer's dementia
- ☐ Reduced frequency of social contacts associated with increased risk of all types of dementia
- ☐ Late life depression associated with increased risk of all types of dementia
- ☐ Type 2 diabetes associated with increased risk of vascular dementia
- ☐ Rural living associated with increased risk of Alzheimer's dementia

Your answer is incorrect.

A recent review of systematic reviews and meta-analyses examined associations between 76 factors and dementia. Seven associations presented convincing evidence. These were benzodiazepine use, depression at any age, late life depression, frequency of social contacts for all types of dementia. Type 2 diabetes was associated with vascular dementia and Alzheimer's disease. Obesity has previously been associated with Alzheimer's dementia. Rural living was found to not be significant in terms of association with dementia of any type. (Ref: Bellou V, Belbasis L, Tzoulaki I, Middleton LT, Ioannidis JP, Evangelou E. Systematic evaluation of the associations between environmental risk factors and dementia: An umbrella review of systematic reviews and meta-analyses. *Alzheimer's & Dementia*. 2017 Apr 1;13(4):406-18.)

The correct answer is: Rural living associated with increased risk of Alzheimer's dementia

Question 66

Not answered

Marked out of 1.00

Regarding suicide in later life, which ONE of the following interventions is correctly matched to its type of preventative strategy?

Select one:

- ☐ Restriction of lethal means of suicide – indicated prevention
- ☐ ECT for inpatients with psychotic depression – selective intervention
- ☐ Peer companionship and befriending programs for isolated older people – universal prevention
- ☐ Education program to improve GP's ability to treat depression – universal prevention
- ☐ Proactive screening for depression and pain management in older adults in primary care with poorly controlled pain – selective intervention

Your answer is incorrect.

Universal prevention aims to reduce incidence at whole community or society levels. Selective prevention targets groups who do not currently have suicidal ideation but are vulnerable due to life circumstances or demonstrate risk factors for depression and suicide. Indicated prevention focusses on individuals who are in imminent danger of attempting suicide, have recently done so, or express suicidal thoughts. Restriction of lethal means is a universal prevention. Education programs to improve GP's treatment is a selective prevention as it focusses on patients who are already diagnosed with depression. Peer companionship for isolated older people is a selective intervention as it targets lonely people who are at higher risk of suicide. ECT for inpatients with psychotic depression is an indicated prevention strategy as it targets those who are high risk of suicide. Proactive screening for depression of older adults with uncontrolled pain is a selective intervention as it targets a group who do not necessarily have depression but have pain which is a risk factor for developing depression and attempting suicide. (Ref: Lapierre S, Erlangsen A, Waern M, De Leo D, Oyama H, Scooco P, et al. A systematic review of elderly suicide prevention programs. Crisis. 2011;32(2):88-98)

The correct answer is: Proactive screening for depression and pain management in older adults in primary care with poorly controlled pain – selective intervention

Question **67**

Not answered

Marked out of 1.00

Mr Jones is a 65-year-old man who is brought to A&E with a sudden onset of severe headache, vomiting, and dizziness, after having had a brief loss of consciousness whilst in the supermarket. He has a pacemaker and history of hypertension. Which investigation would be the initial priority in his emergency management?

Select one:

- ☐ Lumbar puncture
- ☐ MRI Brain
- ☐ Echocardiogram
- ☐ Chest X-Ray
- ☐ CT Brain

Your answer is incorrect.

CT brain is the most important initial investigation for this gentleman who may have a sub-arachnoid haemorrhage (SAH). The cornerstone of SAH diagnosis is the noncontrast head CT scan. The sensitivity of head CT for detecting SAH is highest in the first 6 to 12 hours after SAH (nearly 100%) and then progressively declines over time to about 58% at day five. MRI is also sensitive but would be contraindicated because of his pacemaker. An echocardiogram is not the most important investigation at this point, nor is a chest X-ray. A lumbar puncture would be appropriate if the CT head was negative and there was still a high suspicion of a subarachnoid haemorrhage. (Ref: Vermeulen M, van Gijn J. The diagnosis of subarachnoid haemorrhage. Journal of Neurology, Neurosurgery, and Psychiatry. 1990;53(5):365-372)

The correct answer is: CT Brain

Question 68

Not answered

Marked out of 1.00

Mr Evans has been referred to secondary care mental health services by his GP. He has read about repetitive Transcranial Magnetic Stimulation and would like to try it. NICE supports the use of rTMS for which one of the following conditions?

Select one:

- ☐ Anxiety
- ☐ Schizophrenia
- ☐ Dementia
- ☐ Depression
- ☐ Obsessive Compulsive Disorder

Your answer is incorrect.

Evidence on repetitive Transcranial Magnetic Stimulation for depression shows no major safety concerns. The evidence on its efficacy in the short term is adequate, although the clinical response is variable. Repetitive Transcranial Magnetic Stimulation for depression may be used with normal arrangements for clinical governance and audit. (Ref: <https://www.nice.org.uk/guidance/ipg542/chapter/1-Recommendations>)

The correct answer is: Depression



Question 69

Not answered

Marked out of 1.00

Mr. Gordon is a 65-year-old man who, over the past 2 years, has been making simple mistakes in his work as an accountant and has now started to impulsively shout at his colleagues and appears very 'moody' in response to minor irritations. He does not believe there is anything wrong with his behavior. Which of the following is most likely to be the diagnosis?

Select one:

- ☐ Lewy body dementia
- ☐ Frontal lobe dementia
- ☐ Depression
- ☐ Huntington's disease
- ☐ Alzheimer's dementia

Your answer is incorrect.

This is most likely to represent frontal lobe dementia as his social behaviour has coarsened, he is making errors at work, which are likely due to executive dysfunction, and he has no insight into his difficulties. Huntington's is unlikely due to his age and lack of motor symptoms. There is no indication of mood symptoms or memory impairment, nor is there any fluctuation in functioning.

The correct answer is: Frontal lobe dementia

Question 70

Not answered

Marked out of 1.00

Mrs. Hornbeak is an 80-year-old woman with a diagnosis of dementia. Over the last 9 months her family has noted that she has been low in mood and she has recently been diagnosed with a depressive episode. In which of the following dementia types is co-morbid depression most prevalent?

Select one:

- ☐ Alzheimer's dementia
- ☐ Frontotemporal dementia
- ☐ Vascular dementia
- ☐ Huntington's disease
- ☐ Parkinson's dementia

Your answer is incorrect.

Vascular dementia can be associated with depression in around 40-50% of cases. Huntington's disease and Parkinson's dementia are also associated with increased rates of depressive illness.

The correct answer is: Vascular dementia



Question 71

Not answered

Marked out of 1.00

Mrs. Patterson is a 66-year-old woman who has presented with evidence of dementia. She has a personal history of severe migraine and a strong family history of stroke and early onset dementia. Which of the following is the most likely diagnosis? Mrs. Patterson is a 66-year-old woman who has presented with evidence of dementia. She has a personal history of severe migraine and a strong family history of stroke and early onset dementia. Which of the following is the most likely diagnosis? Mrs. Patterson is a 66-year-old woman who has presented with evidence of dementia. She has a personal history of severe migraine and a strong family history of stroke and early onset dementia. Which of the following is the most likely diagnosis?

Select one:

- ☐ CADASIL
- ☐ MELAS
- ☐ Pick's disease
- ☐ Normal pressure hydrocephalus
- ☐ Lewy body dementia

Your answer is incorrect.

CADASIL (Cerebral autosomal dominant arteriopathy with subcortical infarcts and leukoencephalopathy) is an inherited condition caused by abnormalities on the NOTCH-3 gene on the short arm of Chromosome 19. It is characterised by an autosomal dominant pattern of inheritance with affected individuals experiencing migraines and then TIAs and CVAs around the age of 50 onwards. Mitochondrial encephalomyopathy, lactic acidosis, and stroke-like episodes (MELAS) is a condition that affects many of the body's systems, particularly the brain, nervous system and muscles.

The correct answer is: CADASIL

Question **72**

Not answered

Marked out of 1.00

Mr Tetley is an 81-year-old man who has been referred to your outpatient clinic. He and his family have noticed that he is experiencing difficulties with his memory and has started to have unexplained falls with some rigidity. Which of the following is the most likely diagnosis?

Select one:

- ☐ Progressive Supranuclear palsy
- ☐ Parkinsonism
- ☐ Corticobasal degeneration
- ☐ Dementia with Lewy Bodies
- ☐ Multiple system atrophy

Your answer is incorrect.

This is most likely to be dementia with Lewy Bodies in view of the early cognitive impairment and history of falls and rigidity. The other listed disorders do have cognitive impairment as a feature, although it is often later in the course of the illness.

The correct answer is: Dementia with Lewy Bodies

Question **73**

Not answered

Marked out of 1.00

Mr. Owsley is a 60-year-old man who has presented with a number of cognitive impairments and has been diagnosed with a non-degenerative dementia. Which of the following is an example of a non-degenerative dementia?

Select one:

- ☐ Dementia with Lewy bodies
- ☐ Multi-system atrophy
- ☐ Multiple sclerosis
- ☐ Huntington's disease
- ☐ Alcoholic dementia

Your answer is incorrect.

Alcoholic dementia is a non-degenerative process as the damage caused ceases once alcohol is no longer being consumed. The other disorders listed are degenerative and progressive. Other examples of non-degenerative dementia include: cancer with or without brain metastases; normal-pressure hydrocephalus; subdural haematoma; hypothyroidism; vitamin B12 deficiency; and neurosyphilis.

The correct answer is: Alcoholic dementia

Question **74**

Not answered

Marked out of 1.00

Mrs Terry is a 78-year-old woman who has recently been diagnosed with mild Alzheimer's dementia. She is aware of the diagnosis and is keen to start treatment. She has recently been started on Donepezil after the benefits and risks were explained to her. Which of the following physiological parameters is the most important to monitor in her case?

Select one:

- ☐ Temperature
- ☐ Heart rate
- ☐ Oxygen saturation
- ☐ Urinary output
- ☐ Blood pressure

Your answer is incorrect.

Donepezil can be associated with bradycardia, heart block and syncope. Heart rate must be monitored when this medication is started and when the dose is increased.

The correct answer is: Heart rate



Question 75

Not answered

Marked out of 1.00

Mrs Brown is a 72-year-old lady who has recently been diagnosed with lung cancer whilst on the medical ward. The geriatrician looking after her is seeing if it is possible to rationalise her medication from her notes. She has been on a long-term prescription of diazepam 5mg BD from her GP. The indication for this is not clear. Which of the following diagnoses are associated with prescription benzodiazepine use in older people?

Select one:

- ☐ Panic disorder
- ☐ Epilepsy
- ☐ COPD
- ☐ Dementia
- ☐ Schizophrenia

Your answer is incorrect.

According to the below study female sex, mild cognitive impairment, depression, panic disorder and suicidal ideation were associated with benzodiazepine dependence in community dwelling older people. (Ref: Voyer P, Preville M, Roussel ME, Berbiche D, Beland SG. Factors associated with benzodiazepine dependence among community-dwelling seniors, J Community Health Nurs , 2009, vol. 26 (pg. 101-13))

The correct answer is: Panic disorder

Question **76**

Not answered

Marked out of 1.00

The Driving Vehicle Licensing Agency (UK) requests that they are informed if people are diagnosed with certain mental illnesses as these may impact on their ability to drive. Which of the following diagnoses is most commonly associated with dangerous driving?

Select one:

- ☐ Emotionally unstable personality disorder
- ☐ Panic disorder
- ☐ Vascular dementia
- ☐ Bipolar disorder
- ☐ Depression

Your answer is incorrect.

Dementia, except potentially in the very early stages, is associated with significant risks around driving and the DVLA generally advises that people stop driving after they receive a diagnosis. The other disorders listed can be associated with dangerous driving but this is dependent upon the symptoms an individual experience, whether they are experiencing an episode of illness.

The correct answer is: Vascular dementia



Question 77

Not answered

Marked out of 1.00

Mrs Larkin attends the diagnostic memory clinic with her husband. She is 55 years old and currently working as a teacher but has noticed that she is finding it increasingly difficult to plan lessons and has lost her confidence in social situations. Her husband has noticed she is repetitive and is worried because she got lost driving to their holiday house despite usually knowing the route well. What is the most common neurodegenerative disorder in those aged less than 65 years?

Select one:

- ☐ Alzheimer's dementia
- ☐ Alcoholic dementia
- ☐ Huntington's Disease
- ☐ Vascular dementia
- ☐ Frontotemporal dementia

Your answer is incorrect.

The most common cause of young onset dementia (ie: in those aged 30 - 64) is Alzheimer's disease, followed by vascular, followed by frontotemporal, alcohol related, dementia with Lewy bodies, and Huntington's disease. (Ref: Sampson EL, Warren JD, Rossor MN. Young onset dementia. Postgraduate Medical Journal 2004;80:125-139).

The correct answer is: Alzheimer's dementia

Question 78

Not answered

Marked out of 1.00

Mrs Jones is a retired science teacher and her husband has recently been diagnosed with dementia. She is interested to know how his dementia is classified. Which of the following dementia is correctly matched with the brain area it affects in the initial stages of the disease process?

Select one:

- ☐ Frontotemporal dementia - subcortical
- ☐ Dementia with Lewy Bodies - cortical
- ☐ Huntington's disease – cortical
- ☐ Vascular dementia – subcortical
- ☐ Alzheimer's disease – subcortical

Your answer is incorrect.

Alzheimer's disease and frontotemporal dementia are both cortical. Vascular dementia, Progressive Supranuclear Palsy, Huntington's disease, multiple sclerosis, Parkinson's disease and hydrocephalus are sub-cortical. Dementia with Lewy bodies is considered to be cortico-subcortical (Mixed type) (Ref: Shaik, S. S. and Varma, A. R. (2012), Differentiating the dementias: a neurological approach. Prog. Neurol. Psychiatry, 16: 11-18. doi:10.1002/pnp.224 <http://onlinelibrary.wiley.com/doi/10.1002/pnp.224/pdf>)

The correct answer is: Vascular dementia – subcortical

Question 79

Not answered

Marked out of 1.00

The type of family therapy that depends on transactions is:

Select one:

- ☐ Behavioural Couples Therapy
- ☐ Strategic Family Therapy
- ☐ Functional Family Therapy
- ☐ Structural Family Therapy
- ☐ Systemic Family Therapy

Your answer is incorrect.

A family establishes a set of rules for its daily functioning through its transactions. According to structural family therapy, these rules form a family unit's structure.

The correct answer is: Structural Family Therapy



Question 80

Not answered

Marked out of 1.00

Natasha has been referred to the Adolescent Mental Health Team following an episode of self-harm in which she used a razor to superficially lacerate her thighs. On further exploration, she tells you that her boyfriend had recently broken up with her, and she has been feeling terrible about herself. She describes to you her struggles with intense, fluctuating emotions, identity confusion, impulsivity and frequent suicidal ideation. This has been going on for a few years and makes it difficult for her to maintain friendships. She asks you if there are any interventions that have good evidence of helping young people with difficulties similar to her own. You recommend:

Select one:

- ☐ DBT
- ☐ Fluoxetine
- ☐ CAT
- ☐ Interpersonal Therapy
- ☐ Family Therapy

Your answer is incorrect.

Even though there is some evidence of efficacy in the treatment of features of Borderline Personality Disorder with other therapies, the strongest evidence for its use in adolescence is for Dialectical Behavioural Therapy where it is associated with an improvement in suicidality, non-suicidal self-injurious behaviour, emotional dysregulation and depression from the beginning of the therapy to the one-year follow-up (Child and Adolesc Psych and Ment Health 2011; 5:3)

The correct answer is: DBT

Question **81**

Not answered

Marked out of 1.00

Mrs Galloway's husband died of lung cancer 12 months ago. He had been severely unwell for two months prior to his death. She has come to see you and describes her struggles to you. Which of the following features is most suggestive of depression?

Select one:

- ☐ Anger and irritability
- ☐ Preoccupation with the deceased
- ☐ Denial
- ☐ Excessive guilt feelings
- ☐ Visual hallucinations of her husband

Your answer is incorrect.

An excessive guilt feeling is most suggestive of depression.

Feelings of guilt can be normal, after the death of someone close. But if they are strong and prolonged, then it is suggestive of depressive reaction. Visual hallucinations may also be part of normal grief, but if they are frequent, persistent, and disruptive to functioning then a psychotic illness should be considered.

The correct answer is: Excessive guilt feelings



Question 82

Not answered

Marked out of 1.00

Maria is a 22-year-old woman who tells you about her impulse to turn the light switch on and off seventeen times every time she leaves a room. She says that if she doesn't do it, she fears an explosion might happen and her family will be severely injured. Maria says that given that such an explosion hasn't happened, the repetitive behaviour must be working. What defence mechanism is most associated with OCD?

Select one:

- ☐ Positive punishment
- ☐ Denial
- ☐ Positive reinforcement
- ☐ Negative reinforcement
- ☐ Reciprocal inhibition

Your answer is incorrect.

Described by Skinner, and part of operant conditioning, negative reinforcement is a behaviour that is strengthened by avoiding a potential negative outcome. In this case Maria thinks that by switching the light on and off a number of times she is preventing injury occurring to her family.

The correct answer is: Negative reinforcement

Question 83

Not answered

Marked out of 1.00

Mrs Holloways' husband died of cardiac failure 3 months ago. She is grieving for the loss of her husband. Which of the following is a feature of normal grief reaction?

Select one:

- ☐ Visual hallucinations other than the deceased
- ☐ Pining or searching for the deceased
- ☐ Auditory hallucinations other than the deceased
- ☐ Inability to function
- ☐ Strong suicidality

Your answer is incorrect.

Pining or searching for the deceased is a feature of normal grief reaction. Features of atypical grief reaction include; Strong suicidality, Inability to function, visual and auditory hallucinations other than the deceased.

The correct answer is: Pining or searching for the deceased



Question **84**

Not answered

Marked out of 1.00

Your patient reports that her coworker has got several promotions and bonuses, but she has been working in the company longer than her. She appears resentful and is considering quitting her job. Which of the following cognitive biases does she suffer from?

Select one:

- ☐ Selective abstraction
- ☐ Jumping to conclusions
- ☐ Fallacy of fairness
- ☐ Overgeneralisation
- ☐ Fallacy of misattribution

Your answer is incorrect.

Your patient seems to be suffering from the so-called fallacy of fairness. People with this cognitive bias often firmly believe that life is always supposed to be fair, and feel resentful when it turns out not to be fair.

The correct answer is: Fallacy of fairness



Question 85

Not answered

Marked out of 1.00

Miss Jane Lloyd is a 28-year-old woman who has presented with a 10-year history of binge eating followed by self-induced vomiting and inappropriate use of laxatives. She reports difficulties with her body image. She is not currently receiving any treatment. Which of the following would be most appropriate to offer her?

Select one:

- ☐ Supportive counselling
- ☐ CBT
- ☐ Family therapy
- ☐ Fluoxetine
- ☐ Interpersonal therapy

Your answer is incorrect.

NICE guidelines for the treatment of bulimia nervosa recommend that adult patients should be offered a bulimia-nervosa-specific self-help programme or specialised course of CBT. Medication should not be offered as a treatment for bulimia nervosa alone and family therapy is indicated only for child or adolescent patients.

The correct answer is: CBT



Question 86

Not answered

Marked out of 1.00

Miss Anne Gowen is a 23-year-old medical student who is currently revising for her final examinations. She had been feeling very anxious about this but has been attempting to view them as an opportunity to consolidate her knowledge and show off her skills. Which of the following is she most likely to be demonstrating?

Select one:

- ☐ Reaction formation
- ☐ Intellectualisation
- ☐ Sublimation
- ☐ Suppression
- ☐ Reframing

Your answer is incorrect.

This is most likely to be an example of reframing, when an individual attempts to view a potentially anxiety-provoking situation as something positive so it is no longer a source of distress.

The correct answer is: Reframing



Question **87**

Not answered

Marked out of 1.00

Miss Louise Guest is a 42-year-old woman who has been referred to psychodynamic psychotherapy due to a long-standing history of low mood and anxiety. She attends regularly for 2 months and seems to find the sessions difficult although appear to be therapeutically useful. She then writes a letter to her therapist stating that she will not be attending any longer as she feels she no longer requires their input. Which of the following is she most likely to be demonstrating?

Select one:

- ☐ Denigration
- ☐ Isolation
- ☐ Undoing
- ☐ Rationalisation
- ☐ Denial

Your answer is incorrect.

This is most likely to represent rationalisation. The patient has been finding the therapy emotionally challenging and, rather than acknowledge this, subconsciously produces a superficially reasonable explanation to avoid it.

The correct answer is: Rationalisation

Question **88**

Not answered

Marked out of 1.00

Mr Carl Sullivan is a 38-year-old man who has a diagnosis of OCD. He was assessed and referred for CBT. He finds himself being attracted to the therapist and subsequently joins a gym. Which of the following is he demonstrating?

Select one:

- ☐ Repression
- ☐ Sublimation
- ☐ Displacement
- ☐ Regression
- ☐ Rationalisation

Your answer is incorrect.

This is most likely to be sublimation. The patient is experiencing an erotic transference towards the therapist and, rather than acting on it in an inappropriate way, directing it towards a more appropriate activity. This erotic attraction is a form of positive transference, whereby the therapist is viewed as helpful and a good influence but taken further. It is a relatively common phenomenon.

The correct answer is: Sublimation

Question 89

Not answered

Marked out of 1.00

Miss Kiera Bowles is a 20-year-old woman who has presented with difficulties with her body image associated with marked restriction of dietary intake and excessive exercise. Her BMI is currently 17. Which of the following should she be offered as an initial treatment?

Select one:

- ☐ Self help
- ☐ Guided self help
- ☐ CBT
- ☐ Eating-disorder-focused focal psychodynamic therapy
- ☐ Fluoxetine

Your answer is incorrect.

The initial treatment for anorexia nervosa should be one of: eating-disorder-focused cognitive behavioural therapy (CBT-ED), Maudsley Anorexia Nervosa Treatment for Adults (MANTRA), or specialist supportive clinical management (SSCM). Eating-disorder-focused psychodynamic therapy can be offered if these options are unsuccessful or are not acceptable to the patient. Medication should not be used as a sole treatment in anorexia nervosa.

The correct answer is: CBT

Question 90

Not answered

Marked out of 1.00

Mr Davies is a 45-year-old man who is the sole care provider for his daughter who has multiple physical disabilities and requires full-time care. He presents to his local Emergency Department with an inability to move his left leg. He is thoroughly examined and investigated and no physical cause can be found. Which of the following defense mechanisms is he most likely to be using in this case?

Select one:

- ☐ Repression
- ☐ Regression
- ☐ Projective identification
- ☐ Displacement
- ☐ Projection

Your answer is incorrect.

This is most likely to present displacement with the patient's stress and difficulty coping with her daughter's care needs has been converted into a physical symptom that prevents him from functioning at his usual level.

The correct answer is: Displacement



Question 91

Not answered

Marked out of 1.00

Miss Islay is a 40-year-old woman who has been referred to psychology services for input around depressive and anxiety symptoms. She is due to start a therapeutic intervention that involves the use of circumscribed and hypothetical questions. Which of the following psychotherapies uses these approaches?

Select one:

- ☐ Cognitive behavioural therapy
- ☐ Dialectical behavioural therapy
- ☐ Psychodynamic psychotherapy
- ☐ Cognitive analytic therapy
- ☐ Rational emotive therapy

Your answer is incorrect.

Psychodynamic psychotherapy makes use of hypothetical and circumscribed questions to gain a greater understanding of a patient's underlying thought processes and defenses.

The correct answer is: Psychodynamic psychotherapy



Question 92

Not answered

Marked out of 1.00

Various theories have been proposed regarding factors, which predict response to psychotherapy. Which of the following is included in Yalom's curative factors for group therapy?

Select one:

- ☐ Pre-treatment motivation
- ☐ Catharsis
- ☐ Socialising outside the group
- ☐ Positive relationship with therapist
- ☐ Self-disclosure

Your answer is incorrect.

Catharsis - Releases suppressed emotions and promotes healing by disclosing information to group members. It is one of the factors included in Yalom's curative factors for group therapy.

The correct answer is: Catharsis



Question 93

Not answered

Marked out of 1.00

Miss Johnson is a 23-year-old lady who has attended A&E with superficial self-harm wounds to her arms and thighs in the context of a recent breakup. You speak with her about her experiences and take a history. She has presented to A&E in the past with a number of small overdoses and has been self-harming by cutting since she was a teenager. She describes a long history of unstable moods and feeling empty inside however she is not currently suicidal, nor does she have any plans to end her life. She is interested in finding out more about what might cause her symptoms and engage in treatment. What management should she be offered?

Select one:

- ☐ Debriefing
- ☐ Brief interpersonal therapy
- ☐ Ask GP to refer to IAPT (Improving access to psychological therapies)
- ☐ Low dose Quetiapine
- ☐ Mentalisation based therapy

Your answer is incorrect.

It is likely Miss Johnson has either a diagnosis of Emotionally unstable personality disorder (EUPD) or at least traits of emotion instability. In deciding about psychological treatments in EUPD and borderline personality disorder the clinician should take into account the choice of the service user, availability of therapies, willingness to engage. There is good evidence for mentalisation-based therapy in EUPD. Brief psychological interventions (<3 months) should not be used (therefore Brief Interpersonal Therapy is not appropriate), and IAPT via GP is also unlikely to be inappropriate for this reason also. Debriefing is not indicated and drug treatment should not be specifically used for emotionally unstable personality disorder, and would not be first line. (Ref: <https://www.nice.org.uk/guidance/cg78/chapter/1-Guidance>)

The correct answer is: Mentalisation based therapy

Question **94**

Not answered

Marked out of 1.00

Mr. Edmunds is a 32-year-old man who has recently left his job. He reports that he has left several jobs over the last 3 years as "they start to hate me and then I leave". Which of the following defense mechanisms is he most likely to be using?

Select one:

- ☐ Displacement
- ☐ Repression
- ☐ Projective identification
- ☐ Projection
- ☐ Splitting

Your answer is incorrect.

This is most likely to represent projection, with the patient projecting his anger and dislike of work or his colleagues towards them. He then perceives them as disliking him and leaves his work. Projection is often associated with socially unacceptable thoughts and feelings, which an individual cannot integrate into themselves.

The correct answer is: Projection



Question 95

Not answered

Marked out of 1.00

Miss Close is a 34-year-old woman with Down's syndrome. She is relatively independent although she lives with her parents. They are elderly and her mother has expressed concerns about Miss Close developing dementia in the future and what this would mean for her independence. What percentages of people with Down's syndrome have dementia by the age of 35 years?

Select one:

- ☐ 35%
- ☐ 45%
- ☐ 25%
- ☐ 55%
- ☐ 15%

Your answer is incorrect.

Around 25% of people with Down's syndrome will show cognitive impairment, including agnosia and apraxia, by the age of 35 years. Around 75% will develop Alzheimer's dementia by the age of 65 years. Most people with Down's syndrome have evidence of neuronal plaques and neurofibrillary tangles on pathological examination by the age of 40.

The correct answer is: 25%

Question 96

Not answered

Marked out of 1.00

James Norris is a 7-year-old boy who has had difficulties with facial grimacing and vocal tics for around 2 years. He has now presented again with problems with repeated eye blinking and a throat clearing sound. Which of the following is the most likely diagnosis?

Select one:

- ☐ ADHD
- ☐ Chronic tic disorder
- ☐ OCD
- ☐ Transient tic disorder
- ☐ Tourette syndrome

Your answer is incorrect.

This is Tourette syndrome which is diagnosed when there have been multiple motor tics and at least one vocal tic present for at least a year. Chronic tic disorder is diagnosed with multiple motor or vocal tics, but not both, have been present for more than a year, while transient tic disorder is diagnosed when tics have been present for at least 4 weeks but less than a year.

The correct answer is: Tourette syndrome



Question 97

Not answered

Marked out of 1.00

Incidence rates of some genetic conditions can have unequal sex distribution. Which of the following conditions have an equal female:male distribution?

Select one:

- ☐ Fragile X syndrome
- ☐ Prader-Willi syndrome
- ☐ Rett syndrome
- ☐ Lesch-Nyhan syndrome
- ☐ Incontinentia pigmenti

Your answer is incorrect.

Prader-Willi syndrome has an equal female:male ratio. Rett syndrome occurs solely in females. Fragile X has a 1:2 female:male ratio. Lesch-Nyhan syndrome occurs almost exclusively in males. Incontinentia pigmenti is seen only in females in clinical practice; it is invariably fatal in male fetuses.

The correct answer is: Prader-Willi syndrome

Question 98

Not answered

Marked out of 1.00

There have been numerous studies into the causes of autism, which have demonstrated that there are several risk factors associated with the condition. Which of the following is the strongest risk factor for an individual developing autism?

Select one:

- ☐ Genetic loading
- ☐ Maternal smoking
- ☐ Socioeconomic status
- ☐ Younger maternal age
- ☐ Small head circumference

Your answer is incorrect.

Genetic loading is the strongest identified risk factor for developing an autism spectrum disorder. Other risk factors include: intellectual impairment; Rett syndrome; Fragile X; immune dysfunction; maternal gestational diabetes; maternal bleeding during pregnancy; maternal medications causing teratogenic effects (especially valproate); antenatal intrauterine infection; foetal presentation; umbilical-cord complications; foetal distress; birth injury or trauma; multiple birth; maternal hemorrhage; summer birth; low birth weight; small for gestational age; low 5 minute Apgar score; meconium aspiration; neonatal anemia; ABO or Rh incompatibility; and hyperbilirubinemia.

The correct answer is: Genetic loading



Question 99

Not answered

Marked out of 1.00

Certain genetic conditions are important causes of intellectual disability. Which of the following is the second most common genetic cause for intellectual disability?

Select one:

- ☐ Fragile X syndrome
- ☐ Down's syndrome
- ☐ Angelman syndrome
- ☐ Prader Willi syndrome
- ☐ William's syndrome

Your answer is incorrect.

Fragile X is the second most common genetic cause for intellectual impairment, occurring in 1/4000 males and 1/8000 females. Down's syndrome is the most common genetic cause for intellectual impairment, occurring in 1/1000 children. Prader Willi occurs in 1/10-30,000 people; Angelman syndrome occurs in 1/12-20,000; William's syndrome occurs in 1/7500-12000 people.

The correct answer is: Fragile X syndrome



Question 100

Not answered

Marked out of 1.00

Mr Quick is a 28-year-old man with a diagnosis of an autism spectrum disorder. He manages these symptoms well but is troubled by other mental health conditions. Which of the following is the most common comorbidity with autism?

Select one:

- ☐ Depression
- ☐ Panic disorder
- ☐ Generalised anxiety disorder
- ☐ Social phobia
- ☐ Tourette's disorder

Your answer is incorrect.

Social phobia is seen in around 17-30% of people with autism spectrum disorders. Around 84% of people with ASD with have some form of anxiety disorder.

The correct answer is: Social phobia



Question **101**

Not answered

Marked out of 1.00

Mr Edward Giles is a 37-year-old man with a diagnosis of treatment resistant schizophrenia. He is taking Clozapine at 350mg daily which has been more effective than his previous antipsychotics but he continues to have some negative symptoms. He has become obese after starting Clozapine and has been found to have elevated cholesterol levels. Which of the following medications would be most appropriate to augment his existing Clozapine?

Select one:

- ☐ Aripiprazole
- ☐ Carbamazepine
- ☐ Amisupride
- ☐ Haloperidol
- ☐ Valproate

Your answer is incorrect.

It would be most reasonable to utilise Aripiprazole in this situation. This has some evidence for treating negative symptoms when patients are already on Clozapine and can be helpful in terms of improving cardiovascular risk factors. Amisulpride has the best evidence for treating residual positive symptoms.

The correct answer is: Aripiprazole

Question 102

Not answered

Marked out of 1.00

Mr Steven Drake is a 25-year-old student who is currently studying for a Masters. He has developed a depressive episode which was treated with Sertraline. He has been taking 200mg daily for 8 weeks. He reports that his mood has improved but that he is only sleeping 3-4 hours per night and is still not enjoying life. He is particularly worried as he has an examination in 4 days. Which of the following would be the most appropriate management plan at this point?

Select one:

- ☐ Change to Mirtazapine
- ☐ Change to Venlafaxine
- ☐ Change to Imipramine
- ☐ Continue Sertraline
- ☐ Add short course of Zopiclone

Your answer is incorrect.

While it is clear that this patient has not fully responded to Sertraline and it would be reasonable to change him to an alternative antidepressant, this would risk destabilising his mental state immediately before an important life event. It would be most reasonable to give a short course of Zopiclone and then review his antidepressant after his examination is complete.

The correct answer is: Add short course of Zopiclone

Question 103

Not answered

Marked out of 1.00

Miss Emma Mayo is a 34-year-old woman with a diagnosis of schizophrenia that is now regarded as treatment resistant after her symptoms did not fully respond to adequate trials of Risperidone or Olanzapine. She is currently receiving an increasing dose of Clozapine but has developed chest pain and ECG abnormalities. Which of the following would be the most appropriate management plan in this case?

Select one:

- ☐ Stop the Clozapine and refer to cardiology
- ☐ Continue the titration of Clozapine with weekly ECGs
- ☐ Continue the current dose of Clozapine until her symptoms resolve
- ☐ Give Propranolol
- ☐ Add Aripiprazole

Your answer is incorrect.

This is likely to represent myocarditis which is a rare but serious side-effect of Clozapine. If this occurs the Clozapine should be immediately withdrawn and the individual urgently referred to cardiology or the medical assessment unit, depending upon their presentation. Withdrawing the Clozapine can result in the individual's mental state deteriorating and so short-term benzodiazepines or an alternative anti-psychotic should be provided.

The correct answer is: Stop the Clozapine and refer to cardiology

Question **104**

Not answered

Marked out of 1.00

Mr. Osborn is a 51-year-old academic scholar who has presented with aggressive behaviour and declining memory over the past 3 months. He has no previous history of similar difficulties. Which of the following is most likely to be the diagnosis?

Select one:

- ☐ Depression
- ☐ Substance dependence
- ☐ Delirium
- ☐ Schizophrenia
- ☐ Dementia

Your answer is incorrect.

This is most likely to represent an early onset dementia. Delirium is less likely due to the duration of symptoms. Depression and schizophrenia are unlikely as they are not characterised by marked memory disturbance in this age group and there is no evidence of mood or psychotic symptoms. Substance dependence would not generally present with memory impairment as a primary symptom.

The correct answer is: Dementia



Question 105

Not answered

Marked out of 1.00

Mr Jones is a 55-year-old man who is admitted for an elective hernia repair, which was supposed to be a day procedure. He had to remain in hospital overnight due to unexpected, significant blood loss. Although he was seen in pre-admission clinic he did not disclose his significant alcohol use (up to 500mL whiskey each day). On the second day of his admission he is confused, agitated, and shaky and appears to be seeing spiders. Which of the following would not form a part of your initial management plan?

Select one:

- ☐ Reassurance and reorientation
- ☐ Haloperidol
- ☐ Pabrinex (IV or IM) high potency thiamine and vitamin B and C
- ☐ Chlordiazepoxide
- ☐ Physical examination to exclude other medical causes of delirium

Your answer is incorrect.

This gentleman is suffering delirium tremens secondary to alcohol withdrawal. He requires urgent parenteral administration of thiamine and B vitamins as well as chlordiazepoxide to manage his withdrawal. He should also be assessed for any other causes of delirium and a key part of the practical management of delirium is reassurance and reorientation. Antipsychotics such as haloperidol are not first line treatment for delirium tremens and may also lower the seizure threshold.

The correct answer is: Haloperidol

Question **106**

Not answered

Marked out of 1.00

Mr Aubrey is a 36-year-old man with a diagnosis of panic disorder. He is unwilling to have talking therapy but is asking about medication options. Which of the following would be most appropriate medication to consider initially for treating panic disorder?

Select one:

- ☐ Mirtazapine
- ☐ Imipramine
- ☐ Phenelzine
- ☐ Sertraline
- ☐ Pregabalin

Your answer is incorrect.

SSRIs like Sertraline are recommended as the first line medication for panic disorder. Other antidepressants could potentially be used.

The correct answer is: Sertraline



Question 107

Not answered

Marked out of 1.00

Terry is a 43-year-old man with bipolar disorder and is on Lithium treatment. He is now presenting with mixed affective state. His recent Lithium level is 0.8. What would be your next step?

Select one:

- ☐ Start antipsychotic medication
- ☐ Reduce the dose of lithium
- ☐ Increase the dose of lithium
- ☐ Start sodium valproate
- ☐ Start antidepressant medication

Your answer is incorrect.

Mixed affective state should be treated as manic episode. The best strategy in this situation is to start him on an antipsychotic medication. Lithium levels are optimal and to increase further could risk toxicity.

The correct answer is: Start antipsychotic medication

Question **108**

Not answered

Marked out of 1.00

Mr. Jameson is a 35-year-old man who has recently been diagnosed with ADHD of moderate severity. He is keen to receive treatment due to the impact his symptoms have at his work. Which of the following are recommended as the first line treatment in adult ADHD?

Select one:

- ☐ Modafinil
- ☐ Atomoxetine
- ☐ Methylphenidate
- ☐ Bupropion
- ☐ Clonidine

Your answer is incorrect.

Medication is the mainstay of treatment for adults with ADHD. Methylphenidate is the first line treatment, with atomoxetine and dexamphetamine being options if this is ineffective. CBT can be trialled if medication does not show positive effects.

The correct answer is: Methylphenidate



Question 109

Not answered

Marked out of 1.00

Mrs. Kelvin is a 42-year-old woman with a diagnosis of schizophrenia. She has struggled to take her oral medication in the past and has now agreed to have a depot instead. Which of the following depot mediations requires at least 2 hours monitoring after it has been given?

Select one:

- ☐ Zuclopenthixol
- ☐ Risperidone
- ☐ Fluphenazine
- ☐ Paliperidone
- ☐ Olanzapine

Your answer is incorrect.

Patients receiving olanzapine as a depot must be monitored for at least 2 hours after administration. Olanzapine post-injection syndrome has been estimated to occur after 0.07% of olanzapine depot injections and in approximately 1.4% of treated patients. Post-injection syndrome includes a range of signs and symptoms such as sedation and delirium that are consistent with olanzapine overdose. Other symptoms include dizziness, weakness, aggression, ataxia, hypertension and seizures. Extrapyramidal symptoms are also reported, such as tremor, dystonia, akathisia, and tardive dyskinesia. It generally starts around 1 hour after administration and resolves after 24-72 hours.

The correct answer is: Olanzapine

Question 110

Not answered

Marked out of 1.00

John is a 29-year-old man with a long history of paranoid schizophrenia and recently started on a new antipsychotic medication. His baseline BP was 130/70. During his weekly review, his blood pressure was 160/90. Which of the following antipsychotic drug is most associated with hypertension?

Select one:

- ☐ Aripiprazole
- ☐ Quetiapine
- ☐ Clozapine
- ☐ Risperidone
- ☐ Olanzapine

Your answer is incorrect.

Most case reports implicate clozapine with some clearly describing normal blood pressure before clozapine was introduced, a sharp rise during treatment and return to normal when clozapine was discontinued. Data available through the committee on safety of medicines yellow card system in the UK indicate that clozapine is the antipsychotic drug most associated with hypertension. There are a small number of reports on Olanzapine, Risperidone, Quetiapine and aripiprazole. (Ref: The Maudsley prescribing guidelines- 11th edition; pg no-122)

The correct answer is: Clozapine

Question **111**

Not answered

Marked out of 1.00

Miss Lodge is a 29-year-old woman who has been referred to an outpatient clinic and brought by her family. She receives an MMSE but performs poorly. She was irritable. She states that she has no time to complete the test and that she needs to speak to the Chief Constable. Which of the following is the most likely diagnosis?

Select one:

- ☐ Alzheimer's dementia
- ☐ Mania
- ☐ Variant-CJD
- ☐ Frontotemporal dementia
- ☐ Huntington's disease

Your answer is incorrect.

The most likely diagnosis is an episode of mania in view of the grandiose delusion she describes and irritability with the assessment process. The other diagnoses would be unlikely in a person of this age, unless there was some evidence to support them.

The correct answer is: Mania



Question 112

Not answered

Marked out of 1.00

Miss Newton is a 36-year-old woman with a diagnosis of treatment resistant schizophrenia. She is receiving an optimum medication regime for her and is currently an inpatient in a rehabilitation unit with a view towards reintegrating her into "normal life". Which of the following will be most likely to help her return to independent living?

Select one:

- ☐ Interpersonal therapy
- ☐ Social skills training
- ☐ Supported employment
- ☐ Pre-vocational training
- ☐ Literacy skills

Your answer is incorrect.

There is significant evidence to indicate that supported employment, particularly Individual Placement and Support, is effective in enabling people with chronic psychosis to function more effectively in the community.

The correct answer is: Supported employment



Question **113**

Not answered

Marked out of 1.00

Mr. Vano is a 24-year-old man who has recently been admitted to a general hospital. Which of the following physical health conditions would be most likely to increase his risk of suicide?

Select one:

- ☐ Asthma
- ☐ Migraine
- ☐ HIV
- ☐ Hepatitis
- ☐ Head injury

Your answer is incorrect.

A history of a traumatic brain injury may increase the subsequent risk of suicide 9-fold. HIV positive status can double the risk of suicide.

The correct answer is: Head injury



Question **114**

Not answered

Marked out of 1.00

Mr Mullen is a 30-year-old soldier who returned from Afghanistan 3 months ago. Towards the end of his tour of duty he witnessed an unexpected attack where his close friend and fellow soldier was shot and killed. He managed to escape but was shot in his left leg in the cross fire and continues to experience pain. He visits his GP as he is concerned about persistent flashbacks of the event and a sense of being on edge all the time. This has been occurring for the last 1 month. Which features would not be consistent with a diagnosis of PTSD?

Select one:

- ☐ Recurring dreams
- ☐ Irritability
- ☐ Difficulty concentrating
- ☐ Symptoms occurring within 6 months of the original trauma
- ☐ Hypersomnia

Your answer is incorrect.

Recurring dreams, difficulty concentrating and irritability are common features of PTSD. Symptoms typically occur within 6 months (though in delayed onset can occur more than 6 months after the trauma). Hypersomnia is not a common feature in PTSD as the presentation is generally one of hyperarousal.

The correct answer is: Hypersomnia

Question **115**

Not answered

Marked out of 1.00

Miss Saunders is a 25-year-old woman with bipolar disorder, who has been referred to your outpatient clinic by her GP with a likely episode of depression of moderate severity. Which of the following symptoms is she most likely to exhibit?

Select one:

- ☐ Agitation
- ☐ Auditory hallucinations
- ☐ Somatoform disorder
- ☐ Nihilistic delusions
- ☐ Overtalkativeness

Your answer is incorrect.

Depression and somatoform disorder are highly co-morbid, with around 50% of patients with depression or anxiety having significant somatoform symptoms. Nihilistic delusions or auditory hallucinations would indicate a diagnosis of psychotic depression, which would therefore be severe. Over talkativeness and agitation are more likely in mania or a mixed affective episode.

The correct answer is: Somatoform disorder



Question **116**

Not answered

Marked out of 1.00

Mr Mathey is a 26-year-old man who has been brought into A&E with fever, confusion, agitation and auditory hallucinations over the last several hours. He has a lumbar puncture as part of his assessment, which demonstrates lymphocytosis, normal glucose and increased protein. Which of the following is the most likely diagnosis?

Select one:

- ☐ Lewy body dementia
- ☐ Huntington's disease
- ☐ Herpes encephalitis
- ☐ CJD
- ☐ CADASIL

Your answer is incorrect.

Herpes encephalitis is the most likely diagnosis in view of the acute onset of symptoms suggesting systemic inflammation and neurological involvement. Patients commonly have marked lymphocytosis, increased red cells, increased protein and low or normal glucose levels in CSF.

The correct answer is: Herpes encephalitis

Question **117**

Not answered

Marked out of 1.00

Mr Davidson is a 37-year-old man who has a diagnosis of bipolar affective disorder and has been admitted to a mental health unit with a mixed affective episode. He had discontinued his previous medication. Which of the following treatments would be most appropriate for managing his current episode?

Select one:

- ☐ Lithium
- ☐ Lamotrigine
- ☐ Gabapentin
- ☐ Topiramate
- ☐ Sodium Valproate

Your answer is incorrect.

Valproate has established effectiveness in mixed affective episodes, as do atypical antipsychotics. Mixed affective episodes do not appear to respond well to Lithium, in contrast to its effectiveness in classical mania. There is no evidence for Lamotrigine in mania or mixed affective episodes.

The correct answer is: Sodium Valproate

Question **118**

Not answered

Marked out of 1.00

Mr Harris is a 42-year-old man with a diagnosis of treatment resistant schizophrenia who is currently stable. He is a heavy smoker and requires a Clozapine dose of 1000mg daily to maintain his serum clozapine level in a therapeutic range. He has been admitted to a general hospital following a generalised seizure in the community. The medical team does not identify any intracranial or systemic cause for his seizure. Which of the following should be done to reduce the risk of further seizure activity?

Select one:

- ☐ Reduce clozapine
- ☐ Add carbamazepine
- ☐ Change antipsychotic to another atypical
- ☐ Add Amisulpride
- ☐ Add sodium valproate

Your answer is incorrect.

In this case, adding sodium valproate would likely be the most appropriate option. It has a broad spectrum of anti-seizure activity and does not have a major impact upon clozapine metabolism. Reducing clozapine dose is an option in some patients but not for this patient as it would likely lead to his serum clozapine level being below the therapeutic range. Carbamazepine induces the enzymes that metabolise clozapine, reducing the serum clozapine level. Adding amisulpride would likely increase the risk of seizure activity, as it will lower the seizure threshold. And if the patient is treatment resistant, they likely require clozapine; changing to an alternative antipsychotic could lead to an increase in psychotic symptoms.

The correct answer is: Add sodium valproate

Question 119

Not answered

Marked out of 1.00

Mrs. Ginner is a 34-year-old woman with a diagnosis of anorexia nervosa. She has been admitted to a general hospital due to concerns about dehydration and profound weight loss. Disturbance of which of the following electrolytes most requires monitoring during the refeeding of profoundly unweight patients?

Select one:

- ☐ Phosphate
- ☐ Chloride
- ☐ Sodium
- ☐ Calcium
- ☐ Potassium

Your answer is incorrect.

While disturbance of all electrolytes can occur in profoundly underweight patients, it is serum phosphate levels that are most important to monitor. Refeeding increases the release of insulin, which requires phosphate, magnesium and potassium. These are usually depleted body-wide. Serum phosphate is also depleted by the formation of phosphate-containing carbohydrate compounds in the muscle and liver. If refeeding is done gradually, these electrolyte disturbances can gradually be corrected. If done too rapidly, the body does not have the time or capacity to respond to the changes in serum electrolytes, leading to profound hypophosphataemia. Cardiac arrhythmias are the most common cause of death from refeeding syndrome, with other significant risks including confusion, coma and convulsions and cardiac failure.

The correct answer is: Phosphate

Question 120

Not answered

Marked out of 1.00

Mr Maloney is a 27-year-old man admitted to the surgical ward following emergency appendectomy. On the first postoperative night, he is seen to be restless and wanting to know if he can leave the ward. When his history is reviewed it is noted he is a heroin user, and usually smokes daily. Which of the following is not an early sign of opiate withdrawal?

Select one:

- ☐ Yawning
- ☐ Anxiety
- ☐ Muscle aches
- ☐ Diarrhoea
- ☐ Insomnia

Your answer is incorrect.

The early signs of opiate withdrawal include agitation, anxiety, muscle aches, increased tearing, insomnia, runny nose, sweating, yawning. Later symptoms of withdrawal include abdominal cramping, diarrhoea, dilated pupils, goose bumps, nausea, vomiting.

The correct answer is: Diarrhoea



Question **121**

Not answered

Marked out of 1.00

Schizophrenia is associated with a variety of clinical symptoms, aside from hallucinations and delusions. This includes cognitive deficits. Which of the following is most likely to be impaired when people with schizophrenia receive cognitive testing?

Select one:

- ☐ Arithmetic
- ☐ Selective attention
- ☐ Praxis
- ☐ Working memory
- ☐ Abstract reasoning

Your answer is incorrect.

Working memory is most likely to be impaired in schizophrenia. This can have effects upon other cognitive functions, such as selective attention and abstract reasoning.

The correct answer is: Working memory



Question 122

Not answered

Marked out of 1.00

Mrs. Manner is 60-year-old woman who has presented to her GP in a distressed state and reports that she has been experiencing a burning sensation in all her limbs. She is examined and has investigations but no physical cause is found. Which of the following is the most likely diagnosis?

Select one:

- ☐ Hypochondriasis
- ☐ Somatoform pain disorder
- ☐ Factitious disorder
- ☐ Munchausen syndrome
- ☐ Body dysmorphic disorder

Your answer is incorrect.

This is most likely to be somatoform pain disorder, which is characterised by chronic pain that does not fit with any recognised clinical syndromes or neurological pathways and is often associated with marked psychological distress. Somatisation disorder most commonly presents with symptoms related to somatic organ systems, such as gastrointestinal, respiratory and cardiac.

The correct answer is: Somatoform pain disorder



Question 123

Not answered

Marked out of 1.00

Mr Lewis is a 24-year-old man with a diagnosis of social phobia. He has been referred to your outpatient clinic for advice on medication, as he does not feel able to engage in psychological interventions at this time. Which of the following medications would be most likely to be beneficial for him?

Select one:

- ☐ Clomipramine
- ☐ Diazepam
- ☐ Pregabalin
- ☐ Sertraline
- ☐ Atenolol

Your answer is incorrect.

SSRIs are most commonly used in social phobia and have the best evidence base. Pregabalin is used for generalised anxiety disorder but does not have a sufficient evidence base for social phobia. Clomipramine could be used for social phobia, but is not preferred; its main use is in OCD if an SSRI is ineffective. Beta-blockers can be used for performance anxiety and can reduce physical symptoms of anxiety in a given situation but are not recommended for social phobia. Benzodiazepines can be used to manage severe anxiety but do not manage the underlying beliefs and are dependence-forming; they are not generally recommended.

The correct answer is: Sertraline

Question **124**

Not answered

Marked out of 1.00

Miss Megan is a 39-year-old woman who has a history of presenting to various doctors, including GPs, out-of-hours services and private clinics. She is insistent that she has multiple sclerosis and that she is investigated for this despite there being no evidence of this during history taking or examination. When she is reassured that there is no reason for investigations and there is no evidence of MS she tends to become angry before leaving the clinic. Which of the following is most likely to be the diagnosis?

Select one:

- ☐ Hypochondriasis
- ☐ Body dysmorphic disorder
- ☐ Emotionally unstable personality disorder
- ☐ Factitious disorder
- ☐ Somatisation disorder

Your answer is incorrect.

This is most likely to represent hypochondriasis. In this disorder an individual has an overvalued idea that they are ill, often with a specific disorder, and repeatedly presents to health services to confirm this. They can be temporarily reassured in some cases, but can be irritable if healthcare staffs do not accede to their requests.

The correct answer is: Hypochondriasis

Question 125

Not answered

Marked out of 1.00

Mr Powell is a 55-year-old man. He has been picked up by the police and taken to local Emergency Department as being found a substantial distance from his home with no memory of going there. There is no physical cause found for this and his concentration and cognition are intact on testing in the hospital. Which of the following is the most likely diagnosis?

Select one:

- ☐ Depression
- ☐ Dissociation
- ☐ Dementia
- ☐ Delirium
- ☐ Schizophrenia

Your answer is incorrect.

This appears to represent a dissociative fugue whereby an individual, usually after encountering a stressor, will wander in the community until they are found or they come out of the fugue. They will have no memory of their travels although they may have appeared normal to those they encountered. Their cognition is usually normal once the episode has passed.

The correct answer is: Dissociation



Question **126**

Not answered

Marked out of 1.00

Lithium is a highly effective medication for bipolar disorder, which must be maintained within a specific therapeutic range to prevent subtherapeutic levels or toxicity. Which of the following medications is most associated with neurotoxicity when prescribed with lithium?

Select one:

- ☐ Haloperidol
- ☐ Sodium Valproate
- ☐ Aripiprazole
- ☐ Lamotrigine
- ☐ Sertraline

Your answer is incorrect.

Haloperidol, fluphenazine and flupenthixol have been associated with irreversible toxic encephalopathy when combined with lithium, and are particularly associated with neurotoxicity, even when lithium is within the therapeutic range.

The correct answer is: Haloperidol



Question **127**

Not answered

Marked out of 1.00

Mrs. Galsworthy is a 32-year-old woman with a diagnosis of OCD. She is currently under your care in an outpatient clinic and is due to start active treatment. She and her husband have asked about her likely prognosis. Which of the following is most associated with a positive prognosis in the short term in OCD?

Select one:

- ☐ Good response to SSRI
- ☐ No precipitating factor identified
- ☐ Association with hoarding behaviours
- ☐ Supportive family
- ☐ Earlier age of onset

Your answer is incorrect.

An early and positive response to SSRI treatment is predictive of remission of OCD symptoms.

The correct answer is: Good response to SSRI



Question **128**

Not answered

Marked out of 1.00

Mrs. Vaughn is a 38-year-old woman with a diagnosis of epilepsy. She has recently presented to her GP with symptoms of low mood, inability to enjoy her usual activities and poor sleep. Which of the following is most likely to increase the risk of developing depression among people with epilepsy?

Select one:

- ☐ Family history of epilepsy
- ☐ Generalised seizures
- ☐ Early age at diagnosis
- ☐ Complex partial seizures
- ☐ Valproate use

Your answer is incorrect.

Complex partial seizures and absence of secondary generalized tonic-clonic seizures are risk factors for depression. Treatment with clonazepam, frequent hospitalizations (drug resistance) and lack of occupational activity are additional significant contributing factors.

The correct answer is: Complex partial seizures



Question 129

Not answered

Marked out of 1.00

Jake is a 12-year-old boy with a diagnosis of conduct disorder. He has been assessed to have normal intelligence and does not have ADHD. His family have completed a course of multi-systemic therapy with a small benefit. However, he continues to initiate fights at school, has tortured the family dog and recently lit a fire in a rubbish bin outside his home. Which ONE of the following medications would be indicated?

Select one:

- ☐ Methylphenidate
- ☐ Olanzapine
- ☐ Quetiapine
- ☐ Risperidone
- ☐ Atomoxetine

Your answer is incorrect.

Risperidone is licensed for the short-term management of severely aggressive behaviour in young people with conduct disorder who have not responded to psychosocial interventions. Quetiapine and olanzapine are not licensed. Atomoxetine or methylphenidate would be appropriate if he had ADHD but he does not have this diagnosis. (Ref: Antisocial behaviour and conduct disorders in children and young people: recognition and management. Clinical guideline [CG158] <https://www.nice.org.uk/guidance/CG158/chapter/1-Recommendations#pharmacological-interventions-2>).

The correct answer is: Risperidone

Question 130

Not answered

Marked out of 1.00

During your CAMHS rotation you become interested in strategies to prevent depression in school age young people. You decide to join a local project which is offering group training on coping strategies and mindfulness to all year 9 students, with additional 1:1 support for those who are identified to have risk factors for depression. This is an example which of the following types of prevention?

Select one:

- ☐ Selective prevention
- ☐ Universal prevention
- ☐ Universal with indicated prevention
- ☐ Indicated prevention
- ☐ Universal with selective prevention

Your answer is incorrect.

This intervention targets all students (universal) and then students who are deemed to be at higher risk for developing depression (selective). Universal interventions target the general public or a whole population group. Selective interventions focus on a particular group or subgroup who have biological, psychological or social risk factors for developing a mental disorder. Indicated prevention targets high-risk individuals with detectable symptoms but whom do not yet meet the criteria for the disorder. (Ref: Weissberg RP, Kumpfer KL, Seligman ME. Prevention that works for children and youth: An introduction. American Psychological Association; 2003 Jun).

The correct answer is: Universal with selective prevention

Question **131**

Not answered

Marked out of 1.00

Bipolar disorder tends to have an earlier age of onset than unipolar disorder. How much earlier do symptoms typically begin to become evident?

Select one:

- ☐ 10-14 years
- ☐ 5-9 years
- ☐ 15-19 years
- ☐ 1-4 years
- ☐ 20-25 years

Your answer is incorrect.

The average age of onset for bipolar affective disorder is around 6 years prior to that of unipolar depression.

The correct answer is: 5-9 years



Question **132**

Not answered

Marked out of 1.00

Miss Fletcher is a 12-year-old girl with a history of anorexia. She is seen in her local CAMHS clinic and referred for a psychological intervention, which uses mirroring and circular discussion. Which of the following therapies is it most likely to be?

Select one:

- ☐ Strategic family therapy
- ☐ Structural family therapy
- ☐ Cognitive analytical therapy
- ☐ Rational emotive therapy
- ☐ Systemic family therapy

Your answer is incorrect.

Mirroring and circular discussion are techniques used in systemic family therapy which works by identifying and addressing unhelpful patterns of thinking and behaviour in groups of people, including families.

The correct answer is: Systemic family therapy



Question 133

Not answered

Marked out of 1.00

In children, which of the following mental health illnesses is most likely to be co morbid with another mental health illness?

Select one:

- ☐ Depression
- ☐ OCD
- ☐ Specific phobia
- ☐ Social phobia
- ☐ Generalized anxiety disorder

Your answer is incorrect.

Depression in children and adolescents is highly comorbid with other disorders, particularly anxiety. The converse is often true as well, but not to the same extent. According to Cummings et al. "Among youth with depression, anxiety disorders are the most common comorbid mental health disorders, with comorbidity estimates ranging from 15% to 75%. In youth with anxiety disorders, rates of comorbid depressive disorders are generally lower, ranging from 10% to 15%". Ref: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4006306/>

The correct answer is: Depression

Question 134

Not answered

Marked out of 1.00

Mr. Olney is a 15-year-old boy who is smoking a large amount of cannabis on a daily basis. He is unable to engage with education and his family has been unable to encourage him to reduce his use or stop. Which of the following psychological interventions is most likely to be most beneficial?

Select one:

- ☐ Family therapy
- ☐ Interpersonal therapy
- ☐ Psychodynamic psychotherapy
- ☐ Motivational interviewing
- ☐ Psychoeducation

Your answer is incorrect.

Motivational interviewing and motivational enhancement with CBT are effective in reducing cannabis misuse in adolescents.

The correct answer is: Motivational interviewing



Question 135

Not answered

Marked out of 1.00

Brad is a 12-year-old boy with a diagnosis of an autism spectrum disorder. He has a co-morbid intellectual disability and is in a special school for his secondary education. Which of the below is the most accurate estimate of the prevalence of intellectual disability amongst people with a diagnosis of an autism spectrum disorder?

Select one:

- ☐ 40-50%
- ☐ 30-40%
- ☐ 50-60%
- ☐ 10-20%
- ☐ 20-30%

Your answer is incorrect.

There are a wide variety of estimates from 40-69% to 75%, regarding the proportion of people with ASD who have a clinically significant degree of intellectual impairment. However, it appears clear that most people with ASD have a degree of intellectual impairment, with this being more likely in those with autism rather than other spectrum disorders.

The correct answer is: 50-60%



Question **136**

Not answered

Marked out of 1.00

Miss Farnham is a 14-year-old girl who has been referred to your outpatient clinic after her mother took her to the GP with concerns about her weight. She presents as very thin and it is clear that she been restricting her dietary intake for several months. Which of the following would be most important to perform as part of her assessment?

Select one:

- ☐ Admit to Paediatric ward
- ☐ Provide reassurance
- ☐ Food diary
- ☐ Measure blood pressure
- ☐ Weight and height for BMI

Your answer is incorrect.

It appears that this patient has anorexia nervosa. It is essential to establish a BMI to establish the diagnosis and to determine the likelihood of physiological compromise. A blood pressure, ideally lying and standing, would be useful, as would some targeted investigations, particularly measurements of electrolytes and serum haemoglobin.

The correct answer is: Weight and height for BMI



Question 137

Not answered

Marked out of 1.00

Miss Jones is a 13-year-old girl with a diagnosis of severe intellectual disability. She has been prescribed a psychotropic medication but her mother believes that this has caused her difficulties in swallowing adequately. Which of the following medications would be most likely to cause this side effect?

Select one:

- ☐ Paroxetine
- ☐ Olanzapine
- ☐ Lamotrigine
- ☐ Fluoxetine
- ☐ Zopiclone

Your answer is incorrect.

Olanzapine would be the most likely medication of this list to cause dysphagia due to its sedative effect and its blockade of dopamine that can lead to a parkinsonian impairment of swallowing. Benzodiazepines like clonazepam can also produce this effect.

The correct answer is: Olanzapine



Question **138**

Not answered

Marked out of 1.00

Shaun Richards is a 16-year-old boy who has been brought into his local Emergency Department by his father. He had assaulted his mother at home. He has refused to speak to the police and to healthcare staff. He is currently being supervised by security with no issues. Which of the following would be the most appropriate next step?

Select one:

- ☐ Transfer to custody
- ☐ Urine drug screen
- ☐ Collateral history
- ☐ Physical examination
- ☐ Risk assessment

Your answer is incorrect.

The most appropriate first step would be speaking to the boy's father to obtain a collateral history as no direct history is available. This would then inform any further decisions.

The correct answer is: Collateral history



Question 139

Not answered

Marked out of 1.00

Jack Young is a 15-year-old boy with a diagnosis of ADHD. He is currently maintained on methylphenidate. He and his mother have asked in clinic about what could have caused his ADHD. Which of the following is the most important antenatal risk factor for developing ADHD?

Select one:

- ☐ Maternal caffeine use
- ☐ Maternal smoking
- ☐ Older paternal age
- ☐ High maternal folate levels
- ☐ Maternal SSRI use

Your answer is incorrect.

Antenatal maternal smoking is associated with the development of ADHD in offspring. SSRI use is not associated with ADHD, although bupropion use is; this may be due to its use in smoking cessation. There is contradictory evidence around maternal caffeine use and ADHD. Low maternal folate levels are associated with ADHD. Paternal age is not associated with ADHD.

The correct answer is: Maternal smoking



Question **140**

Not answered

Marked out of 1.00

Jenny is a 3-year-old girl with global developmental delay. She did learn to speak a few words initially but has lost this ability now. She demonstrates breathing difficulties, odd hand movements and also suffers seizures. Which of the following disorders cause epilepsy in childhood?

Select one:

- ☐ Down Syndrome
- ☐ DiGeorge (22q11.2 deletion) syndrome
- ☐ Boerhaave syndrome
- ☐ Rett Syndrome
- ☐ Gilbert Syndrome

Your answer is incorrect.

Rett syndrome is a neurodevelopmental disorder caused by mutations in MECP2 that occurs almost exclusively in females. After a period of initially normal development during the first 6 to 18 months of life, girls experience loss of speech and purposeful hand use, and develop stereotypic hand movements, and gait abnormalities. Epilepsy affects the majority of patients with the syndrome. DiGeorge syndrome affects the palate and heart. Gilbert Syndrome causes benign raised bilirubin levels. Boerhaave syndrome occurs when vomiting causes oesophageal tears. Down Syndrome affects a number of organ systems but is not associated with seizures.

The correct answer is: Rett Syndrome

Question **141**

Not answered

Marked out of 1.00

Mrs Atkins is a 43-year-old woman with a current history of alcohol dependence who is drinking around 500ml of vodka daily. She has recently developed an infection and requires antibiotics from her GP. Which of the following antibiotics should be avoided in her case?

Select one:

- ☐ Nitrofurantoin
- ☐ Trimethoprim
- ☐ Amoxicillin
- ☐ Metronidazole
- ☐ Clindamycin

Your answer is incorrect.

Metronidazole inhibits aldehyde dehydrogenase and causes acetaldehyde to accumulate in the bloodstream. A disulfiram like reaction occurs when consuming alcohol and the patient may complain of symptoms such as facial flushing, throbbing headache and palpitations. A patient taking metronidazole orally should abstain from drinking alcohol while taking metronidazole and for 48 hours after stopping it.

The correct answer is: Metronidazole

Question **142**

Not answered

Marked out of 1.00

Mr Bailey is a 42-year-old man who has a history of alcohol dependence. He has been admitted to a general hospital as an emergency. He is tremulous, sweating profusely and has a marked tachycardia. He has a generalised seizure immediately after admission. Which of the following treatments would be most appropriate?

Select one:

- ☐ Baclofen
- ☐ Antipsychotics
- ☐ Lorazepam
- ☐ Clonazepam
- ☐ Pabrinex

Your answer is incorrect.

This patient is experiencing an alcohol withdrawal state associated with generalised seizures. He requires benzodiazepines to manage the withdrawal symptoms and reduce the risk of further seizure activity. Antiepileptic medications can be used to reduce the risk of seizures but would not address the overall withdrawal state.

The correct answer is: Lorazepam



Question **143**

Not answered

Marked out of 1.00

Miss Sloane is a 31-year-old woman with a long history of opiate dependence. She is currently injecting and smoking heroin but is now motivated to stop and has been engaging in groups delivered via her local drug misuse service. She is now keen to have medications to aid her in stopping heroin use completely and not to feel 'drugged'. Which of the following medications would be most appropriate for opiate detoxification and subsequent treatment?

Select one:

- ☐ Dihydrocodeine
- ☐ Naltrexone
- ☐ Baclofen
- ☐ Bupropion
- ☐ Buprenorphine

Your answer is incorrect.

Buprenorphine or methadone are the first line treatment in opioid detoxification and are most commonly used. Methadone is a full opiate agonist while buprenorphine is a partial agonist, more appropriate when the patient prefers not to have full agonistic effects. Disulfiram is a treatment for alcohol addiction. Naltrexone is an opioid antagonist and should not be used in the initial stages of detoxification. Naltrexone has some potential preventative use following opiate detoxification. Clonidine and dihydrocodeine should not be routinely used for deaddiction purposes.

The correct answer is: Buprenorphine

Question 144

Not answered

Marked out of 1.00

Mrs Lawson is a 52-year-old lady who has suffered from panic disorder with agoraphobia for many years. Since being able to order groceries online she has started drinking up to a bottle of wine each day. She is taken to A&E after her neighbour finds her on the front step having fallen and hit her head. It is likely she was intoxicated at the time of the fall. It is clarified that she doesn't have a subdural haemorrhage but is found to have a probable aspiration pneumonia and is admitted to the medical ward. She begins to withdraw from alcohol on the second day of admission. Despite her heavy drinking her liver function is normal. She is slightly dehydrated with raised urea but her creatinine is within normal limits. Which of the following would be appropriate for the purposes of reducing the risk of seizures associated with Mrs Lawson's abrupt cessation of alcohol?

Select one:

- ☐ Haloperidol
- ☐ Alprazolam
- ☐ Chlordiazepoxide
- ☐ Oxazepam
- ☐ Pabrinex

Your answer is incorrect.

Alprazolam is a very short acting benzodiazepine and is not appropriate for alcohol withdrawal, nor is it readily available in the United Kingdom. Oxazepam is sometimes used in alcohol withdrawal when liver function is impaired but is not first line in someone with normal liver function. Chlordiazepoxide is the first line treatment for management of the GABA receptor sensitivity and glutamate activation due to nervous system hyperactivity (in alcohol withdrawal) and to reduce the risk of seizures. Pabrinex is vitally important for reducing the risk of Wernicke's but is not related to reducing the risk of seizures. Haloperidol is an antipsychotic, which is not first, line treatment in alcohol withdrawal and also lowers seizure threshold.

The correct answer is: Chlordiazepoxide

Question 145

Not answered

Marked out of 1.00

John Masters is a 45-year-old man with cirrhosis secondary to hepatitis C and chronic alcohol dependence. He is admitted to hospital with confusion and is found to have decompensated liver disease and encephalopathy. He is agitated and is refusing medications. Which of the following medications would be appropriate to treat alcohol withdrawal in this gentleman?

Select one:

- ☐ Disulfiram
- ☐ Diazepam
- ☐ Chlordiazepoxide
- ☐ Oxazepam
- ☐ Lorazepam

Your answer is incorrect.

Usually chlordiazepoxide is the agent of choice for treating alcohol withdrawal. However, in patients with cirrhosis or advanced liver disease the shorter half-life of lorazepam and absence of active metabolites with oxazepam may prevent prolonged effects and accumulation and make these better choices. Lorazepam is the correct response to this question as it is available IM if needed (the patient is refusing oral medication at this point). Diazepam is also available IM but has a long half-life. Disulfiram is a treatment for alcohol addiction but is not appropriate in the acute withdrawal state. (Ref: <https://www.uptodate.com/contents/management-of-moderate-and-severe-alcohol-withdrawal-syndromes>)

The correct answer is: Lorazepam



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